Organization, Operation, and Procedures of the Ethical and Administrative Evaluation System for Research in Human Health in Cameroon

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FORWARD

Research plays an essential role in improving populations' access to care. It produces evidence on the distribution and determinants of health problems and provides decision-makers with information to guide their choices. It thus

contributes to improving the performance of the health system. However, it must be done in compliance with the rules of ethics and protection of participants.

By Decree No. 2002/209 of August 19, 2002 organizing the Ministry of Public Health, revised by Decree No. 2013/093 of April 3, 2013, the Government established the Division of Operational Research in Health. More recently, Law No. 2022/008 of April 27, 2022 relating to Medical Research involving Humans, promulgated by the President of the Republic, completed the process of supervising human health research in Cameroon.

Although real progress in the development of 1a. regulation of health research ethics have been accomplished, the decentralization of ethics committees at the regional level and within health institutions requires harmonization of operational procedures for evaluation and ethical and administrative monitoring of research projects. research.

This Guide entitled "Organization, operation and procedures of the ethical and administrative evaluation system of human health research in Cameroon" is an important working tool made available to all stakeholders involved in human health research.

To this end, I invite you to appropriate it and make good use of it for the benefit of research policy as well as the beneficiaries of the conclusions of said research

Ministre de la Santé Publique

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AAR	Administrative Research Authorization
AMM	Marketing Authorization
ANRPh	National Pharmaceutical Regulatory Authority

CCSSRS	Scientific and Strategic Advisory Commission for Health Research
CERSH	Research Ethics Committee for Human Health
CIERSH	Institutional Research Ethics Committees for Human Health
CIOMS	Council for International Organizations of Medical Sciences
CNERSH	National Research Ethics Committee for Human Health
CRERSH	Regional Research Ethics Committees for Human Health
DPML	Directorate of Pharmacy, Medicines and Laboratory
DROS	Division of Operational Research in Health
DSF	Family Health Department
DSMB	Data Safety Monitoring Board
EC	Clinical test
SAE	Serious Adverse Event
IP	Principal investigator
LANACOME	National Laboratory for Quality Control of Medicines and Expertise
MINSANTE	Minister of Public Health
MTA	Improved Traditional Medicines
ONSP	National Public Health Observatory

► CHAPTER I: GENERAL

Health research is defined as a "methodical study aiming to enrich generalizable knowledge" [1]. According to the Order relating to the creation, organization and operation of Research Ethics Committees for Human Health within structures under the Ministry of Public Health in Cameroon [2], research for human health is understood as: ".... the production of knowledge with the aim of understanding health problems and better remedying them……". It encompasses any intervention, collection and analysis of data involving humans. It is interested in pharmaceutical products, diagnostic and therapeutic tools and procedures, personal data, medical records, biological samples, as well as the environment and products intended for consumption. Several actors intervene and collaborate in the landscape of human health research in Cameroon,

In the Health Sector Strategy 2016-2027 [3], health research is a key component of Health System Strengthening (strategic axis No. 4) which must contribute to achieving specific objective No. 4.5 which is "by 2027, ensure the development of health research and the availability of quality health information for decision-making based on evidence at all levels of the health pyramid. This involves strengthening the capacities of institutions and stakeholders in the following areas: seeking funding for research, establishing a legislative and regulatory framework, and coordinating and disseminating research results. On another level, research activities will be primarily focused on the pillars of the health system and medicinal plants for the development of local production of medicines.

1.1 GUIDELINES FOR THE PROTECTION OF PARTICIPANTS IN HUMAN HEALTH RESEARCH IN CAMEROON

Guideline 1: Guiding principles for the submission, evaluation and monitoring of the Ethical Clearance request

The submission, evaluation and monitoring of the implementation of research projects for human health are governed by compliance with the following principles:

- The social benefit of the Cameroonian population for any research project implemented in Cameroon;
- The submission of any research protocol involving humans to evaluation and monitoring by a competent ethics committee;
- The declaration of conflict of interest by members of the ethics committee involved in the evaluation and monitoring of research protocols;
- Scientific validity;
- Minimizing the risks to which the research protocol exposes participants and maximizing the social benefit expected from them;
- The autonomy of research participants by ensuring that the free and informed consent of all participants is obtained before their inclusion in the research and during it;
- The protection of vulnerable participants or those who cannot give free and informed consent at the time of research;
- Equitable access of populations to research and the benefits thereof, including vulnerable populations;
- Protection of data confidentiality and the privacy of research participants;
- Respect for the norms, cultural and social values of households and communities, when planning and implementing research projects;
- Prevention, care and repair of damage suffered by any research participant, even after the research;

- Compliance with local and international regulations when planning and implementing research projects;
- Transparency and fairness in the management of research data and biological samples resulting from research between research teams;
- Good governance in local research teams and in those involving foreign researchers;
- Strengthening the capacities of local teams during the planning and implementation of research projects;
- Respect for intellectual property rules.

Directive 2: Institutions and people who can carry out a human health research project in Cameroon

Human health research in Cameroon is carried out by:

- University institutions;
- Research institutions;
- Health Training;
- Technical Directions: and programs of the Ministry of Health;
- Governmental and non-governmental institutions;
- Independent researchers.

Directive 3: Ethical and administrative itinerary of a research project for human health implemented in Cameroon.

Any research project for human health to be implemented in Cameroon

must follow the following ethical and administrative steps:

- Obtaining agreement in principle from the head of the locality or research implementation sites;
- Subscription to an insurance policy for phase 1, 2,3 clinical trials;
- Signature of a biological material transfer agreement (material transfer agreement) between stakeholders, for research projects requiring

sharing of biological material with institutions outside Cameroon or with local institutions;

- Signature of a data sharing agreement (data sharing agreement) between stakeholders, for research projects requiring data sharing with institutions outside Cameroon or with local institutions;
- Obtaining a special import exemption from the Ministry of Health if it is a study involving the use of products not approved in Cameroon for clinical trials or a study evaluating non-approved products;
- Prior obtaining of Ethical clearance from a competent ethics committee before any implementation of the research protocol;
- Prior obtaining of Administrative Research Authorization from the Ministry of Health before any implementation of the research protocol;
- Notification of adverse events within regulatory deadlines to the Independent Data Monitoring Committee, to the Ethics Committee, to the Management of Operational Research in Health (DROS) and the Directorate of Pharmacy, Medicines and Laboratories (DPML);
- Transmission of progress and final research reports to the ethics committee and to the Ministry of Public Health;
- Return of research results to health authorities including communities with the involvement of DROS;
- Continuity of patient care at the end of clinical trials if the product tested proves beneficial for the health of the participants.

Directive 4: Sites for implementing research projects for human health

Research projects for human health can be implemented in Communities, Health Facilities and other appropriate institutions.

Phase 1 of clinical trials can only be implemented in a first, second and third category hospital, or in an institution dedicated to clinical research with an appropriate technical platform.

Guideline 5: Roles of actors involved in research for human health in Cameroon

From the Ministry of Public Health

The role of the Ministry of Public Health is to:

- Develop and update the guidelines and procedures relating to the protection of participants in human health research;
- Implement, monitor and evaluate the ethics committees for human health research (CERSH);
- Ensure compliance with regulations regarding human health research;
- Ensure the creation and updating of a directory of researchers and a register of human health research projects having obtained Administrative Research Authorization;
- Ensure the restitution and use of the results of research projects to improve the population's access to health care and care protocols;
- Ensure the registration of clinical trials in a recognized national or international site;
- Ensure the protection of the individual data of research participants;
- Participate in building the capacities of actors involved in research.

Ethics committee for Research for Human Health

The role of human health research ethics committees is to:

- Safeguard the dignity, rights, safety and well-being of all current or potential research participants;
- Develop and update guidelines and procedures relating to the protection of human health research participants;
- Ensure compliance with regulations regarding human health research;
- Create and update a directory of researchers and a register of human health research projects that have obtained ethical clearance;
- Ensure the protection of the individual data of research participants;
- Participate in the knowledge of the capacities of the actors involved in the research;
- Establish, monitor and evaluate the CERSH.

Promoters of research for human health

The promoter is a natural or legal person responsible for launching, managing and financing a research project. To this end, he is responsible for:

- Ensure the development of protocols and tools for implementing the research project;
- Ensure that the research site is adequate and the research teams are competent for the proposed research protocol;
- Ensure compliance with ethical rules during the implementation of the research protocol;
- Ensure the monitoring and audit of clinical trials;
- Ensure the establishment of a "Data Safety Monitoring Board" (DSMB) (data monitoring and monitoring committee for clinical trials);
- Ensure the prevention and coverage of damages that affect any participant during and after the clinical trial;

- Ensure the protection of the rights of researchers participating in research;
- Strengthen the capacities of its researchers involved in research;
- Ensure the sharing of data from all sites on the safety of tested health interventions:
- Ensure access for participants to products resulting from research;
- Ensure the restitution of results to stakeholders including communities.

Research institutions and sites

The manager is responsible for:

- Give its agreement in principle for any research project it hosts subject to obtaining Ethical Clearance and Administrative Research Authorization;
- Ensure the respect, dignity, rights, safety and well-being of each participant in the research project implemented within their institution or promoted by said institution;
- Establish internal procedures, where applicable, in collaboration with the principal investigator, for implementation of the research project in accordance with the approved protocol;
- Ensure capacity building of staff involved in research;
- Ensure capacity building of members of the institutional research ethics committee where applicable;
- Ensure the strengthening of the technical platform of your institution by the research promoter.

From the principal investigator

The principal investigator of the research project is any natural person responsible for the scientific conduct of a human health research project.

He is responsible for:

- Obtain ethical clearance and Administrative Research Authorization according to terms defined by regulation, to provide adequate information;
- Obtain free and informed consent from research participants or their legal representatives;
- Guarantee the confidentiality of research data according to the standards of good clinical practice;
- Keep the data collected for at least ten (10) years from the end of research;
- prepare an anonymized progress, annual and final implementation report for the competent administration.
- Ensure that the population categories for which the research results would be useful are eligible;
- Ensure that study sites comply with standards of good clinical practice;
- Proceed with the registration of the clinical trial according to the procedures defined by regulation
- Communicate to the participant information relating to their health during and at the end of the research, publish the results of the research work;
- Report any incident observed in the field to the promoter, to the body in charge of ethics as well as to the administration which issued the Administrative Research Authorization;
- Return the results to stakeholders including communities by imposing the DROS.

Guideline 6: Evaluation deadlines.

The timelines for the ethical evaluation of complete applications for ethical clearance and administrative authorization for research are set as follows.

- 30 days for ethical clearance request;
- 21 days for the Administrative Research Authorization request.

Directive 7: Training of researchers and members of ethics committees

Before being involved in the ethics evaluation, any member of an ethics committee must receive training in the following areas:

- The principles of ethical and scientific evaluation of human health research protocols;
- National and international regulatory texts governing the protection of research participants;
- The functioning of ethics committees in general and in Cameroon in particular;
- Good Clinical Practices.

1.2. RESEARCH ACTORS IN CAMEROON

1.2.1. Stakeholders of the Ministry of Public Health

Decree No. 2002/209 of August 19, 2002 organizing the Ministry of Public Health [4] and revised by Decree No. 2013/093 of April 3, 2013 [5] creates and assigns to the Operations Research Division in Health (DROS) the missions of:

- Monitoring of clinical research studies;
- Promotion of operational research and dissemination of results on the fight against diseases, reproductive health and nutrition, in liaison with the administrations concerned;

- Monitoring questions relating to bioethics, in liaison with the administrations and organizations concerned;
- Translation of convincing research results into action proposals;
- Establishment of scientific networks and promotion of ethics;
- Relations with higher education in the field of initial and continuing training.

These different missions illustrate the multisectoral nature of the implementation and supervision of research for human health.

Within the Ministry of Health (MINSANTE), several stakeholders work in the field of research for human health. These include:

- From the Scientific and Strategic Advisory Commission for Health Research (CCSSRS) [6];
- Bodies of the National Pharmaceutical Regulatory Authority which are the General Inspection of Pharmaceutical Services and Laboratories (IGSPL), the Directorate of Pharmacy, Medicines and Laboratories (DPML), Division of Operational Research (DROS). the Department of Family Health (DSF), the National Laboratory for Quality Control of Medicines and Expertise (LANACOME);
- Technical directorates: Division of Operational Research (DROS), Directorate of the Organization of Care and Health Technology (DOSTS), Directorate of Pharmacy, Medicine and Laboratory (DPML), Directorate of Family Health (DSF), Directorate of the Fight against Diseases, Epidemics and Pandemics (DLMEP), Directorate of Health Promotion (DPS), Directorate of Human Resources (DRJH);
- Priority health programs: National Committee to Fight AIDS (CNLS), National Program to Fight Tuberculosis (PNLT), National Program to Fight Malaria (PNLP), Expanded Vaccination Program (PEY), etc.;

- Laboratories: LANACOME, National Public Safety Laboratory (LNSP), Pasteur Center of Cameroon (CPC), Chantal BIYA International Reference Center for research on the prevention and treatment of HIV/AIDS (CIRCB);
- Health Training courses serving as a site for implementing research for human health;
- Health Districts;
- From the community through dialogue structures: National Public Health Observatory (ONSP), Regional Fund for Health Promotion, District Health Committee, Health Committee, Civil Society Organizations, etc.

There are also several structures and platforms for the dissemination of knowledge generated in the health sector including:

- The Health Information Unit (CIS) responsible for: the design, monitoring and implementation of the health information system, the collection and processing of health statistical data, the establishment of databases relating to public health, the development of national health indicators, the development of National Public Health Accounts, the updating of the health map, the security of statistical data and the publication of health data in conjunction with the ONSP;
- The Health Sector Digital Documentation Center (CDNSS) accessible at the web address www.cdnss.minsante.cm, which is a platform run by DROS aimed at disseminating the results of research work in human health and other documents produced in the health sector;
- The web portal of the National Public Health Observatory (ONSP), available at www.onsp.minsante.cm. which aims to produce and disseminate information on population health in order to inform policies;
- The scientific network Cameroon HIV Research Forum (CAM-HERO) which brings together researchers working on HIV; with the aim of

pooling resources, sharing information and experiences in order to capitalize and promote the results of research on HIV;

- Cameroon Health Research Forum (CAHREF) which organizes every two years, sessions for the dissemination of research results on fundamental and applied research;
- The Center for the Development of Good Health Practices (CDBPS) at the Central Hospital of Yaoundé which documents good health practices and translates them into Strategic Information Notes (NIS).

1.2.2. Other stakeholders

In Cameroon, other structures are involved in the implementation of research activities for human health:

- Ministerial Departments: Ministry of Scientific Research and Innovation (MINRESI), Ministry of Higher Education (MINESUP), Ministry of Agriculture and Rural Development (MINADER), Ministry of Livestock, Fisheries and Animal Industries (MIN EPIA), Ministry of Forests and Wildlife (MINFOF), Ministry of Mines, Industry and Technological Development (MINEP), Ministry of Social Affairs (MINAS), Ministry for the Promotion of Women and the Family (MINPROFF); Ministry of Defense (MINDEF), etc;
- Public or private academic and professional Training institutions: universities (faculties of medicine and biomedical sciences, faculties of science, faculties of human sciences, other specific faculties) and professional training schools (schools of nurses and other health personnel) etc;
- Research institutes and laboratories: National Institute of Statistics (INS), Institute of Demographic Training and Research (IFORD), Institute of Medical Research and Studies of Medicinal Plants (IMPM), Institute of Agricultural Research for Development (IRAD), Center for Research on Emerging and Re-emerging Diseases (CREMER), Research

Center for Armed Forces Health (CRESAR), Central Bureau of Census and Population Study in Cameroon (BUCREP), etc.;

- **Development partners** : World Health Organization (WHO), European and Developing Countries Clinical Trials Partnership (EDCTP), World Bank, United Nations Children's Fund (UNICEF), United Nations Population Fund (UNFPA)), Center for Disease Control and prevention (CDC), World Food Program (WFP), United Nations Development Program (UNDP), Office of the High Commissioner for Refugees (UNHCR), German International Development Cooperation Agency (GIZ) , French Development Agency (AFD), Japanese International Cooperation Agency (JICA), Médecins Sans Frontières (MSF), ICAP of Columbia University, Elizabeth Glaser Pediatric AIDS Foundation (EGPAF), Helen Keller Intonational (HKI), Care International, Plan International Cameroon, Clinton Health Access Initiative (CHAI), Research Institute for Development (IRD), Organization for Coordination and Cooperation for the Fight against Major Endemics in Central Africa (OCEAC), Johns Hopkins Bloomberg School of Public Health, National Agency for Research on AIDS and Viral Hepatitis (ANRS), etc;
- The committees of study: National Research Ethics Committee for Human Health NERSH), Regional Research Ethics Committees for Human Health (CRERSH) and the Institutional Research Ethics Committees for Humahie Health (CIERSH);
- Implementation partners: Cameroon National Association for Family Welfare (CAMNAFAW), Cameroon Baptist Convention Health Service (CBCHS), Horizon Femmes, National Network of Positive Youth (RECAJ+), Cameroon Network of Associations of People Living with HIV/AIDS (RECAP), Care and Health Program (CHP), Council of Heirs International Mission (CHIMISSONS), etc.

1.3. INTERACTIONS BETWEEN STAKEHOLDERS

Collaboration between stakeholders in research for human health is essential. In a spirit of cohesion, it makes it possible to strengthen the protection of subjects. research and community as well as the capacities and protection of research team members. It also contributes to improving compliance with research regulations. These interactions take place both administratively and technically.

1.3.1. Interactions between ethics committees and MINSANTE

-Administratively

Following the decree on the establishment, organization, and functioning of research ethics committees for human health within the entities under the responsibility of the Ministry of Public Health [2], the Minister of Health:

- Establish the ethics committees and determine their composition through decision;
- Appoint by decision the permanent secretary of the national ethics committee of research for human health;
- Allocate a budget and operational funding;
- ensures the external audit of ethics committees;
- ensures the effective application of texts governing ethics committees;
- ensure, through the DROS (Regional Delegation of Public Health), that research projects implemented in Cameroon are evaluated for ethical and scientific aspects by competent and independent ethics committees [7];
- receives from the CNERSH and the CRERSH on a quarterly basis, the session reports and the list of research projects submitted for evaluation.

-**On the technical side**

- The National Ethics Committee, as part of the scientific evaluation of research projects such as clinical trials, verifies that they comply with national and international requirements.

1.3.2. Interactions between ethics committees and researchers/research institutions

In this context, ethics committees must:

- ensure researchers' awareness of harmonized evaluation procedures for research projects;
- ensure ethical monitoring of research;
- ensure collaborative partnership within research teams based on a memorandum of understanding.

1.3.3. Interactions between research teams within the framework of multicenter studies

Seven reference criteria seem essential in multicenter studies for a collaborative partnership. Indeed, it is necessary to ensure that:

- the project is led by a local principal investigator attached to a recognized institution and based in Cameroon:
- those responsible for the implementation structure are informed of the objectives and will be involved in the process of disseminating the results, and where applicable in the follow-up of participants in clinical trials after the research;
- respect and recognition of the distinctive values, culture and social practices of the host community are integrated into the design and implementation of the study
- disparities linked to the research project between researchers, promoters/sponsors and the host community are minimized;
- the community in which the research is carried out is the primary beneficiary of the benefits arising from the implementation of the research;

- databases are accessible to local researchers through data and biological material sharing agreements;
- collaboration also guarantees the transfer of knowledge, technologies and capacity building of local researchers [8].

1.3.4. Interactions between ethics committees and the community

In accordance with guideline 7 of the International Ethical Guidelines for Health Research Involving Human Participants (COIMS), ethics committees are recommended to:

- maintain a file of community actors or community organizations involved in research so that, where applicable, they are involved in the evaluation of certain aspects of the research:
- involve community stakeholders in the design, development and implementation of research, the design and the process of obtaining informed consent from research participants.

1.3.5. Interactions between the different ethics committees (CNERSH - CERSH - CIERSH)

The CNERSH interacts with regional and institutional ethics committees in the following areas:

- support for the selection of their members;
- capacity building of their members;
- monitoring and evaluation of their operation.

Through this collaboration, the CERSH and the CIERSH:

- transmit their reports and minutes of sessions quarterly as well as their annual activity reports to the CNERSH;
- request the expertise of CNERSH as necessary;
- refer to the CNERSH any request to obtain ethical clearance that does not fall within their competence.

1.3.6. Interactions between the Technical Secretariat, the office and the members of the ethics committee

This collaboration [9] is effective through:

- preparation of sessions;
- monitoring the implementation of the Committee's recommendations;
- the transmission of ethical clearance requests;
- administrative and financial management for the operation of the CERSH.

1.3.7 Interactions between ethics committees and Development partners

The interaction between these entities must be limited to technical support, particularly in strengthening the capacities of CERSH members to avoid conflicts of interest.

CHAPTER II: ORGANIZATION AND FUNCTION OF RESEARCH ETHICS COMMITTEES FOR HUMAN HEALTH

The primary responsibility of a Research Ethics Committee for Human Health (CERSH) is to protect research participants. It must also take into account the potential risks and benefits for the community in which a study takes place, its ultimate goal being to promote respect for ethical standards established in human health research. To this end, the CERSH examine research protocols involving human subjects to ensure that they respect locally and internationally recognized ethical principles, and participate in the monitoring of studies during implementation.

II.1. ORGANIZATION OF ETHICS COMMITTEES FOR HUMAN HEALTH RESEARCH

11.1.1. Legal status of CERSH

Research Ethics Committees for Human Health are permanent and independent structures at the central and regional level of the Ministry of Public Health (MINSANTE), research institutions and health facilities.

The operation of these committees does not obey the texts governing that of the working groups of ministerial committees and interministerial committees established by the Decree establishing the modalities for the creation, organization and operation of committees and working groups [10].

The Minister in charge of Public Health represents the various ethics committees (CNESRH, ORERSH, CIERSH) in all acts of civil life. He creates them, and notes the composition of their members. He can take legal action if necessary. If necessary, he can delegate his power of representation to any person or structure of his choice. However, personal or criminal liability may

be incurred for any person or structure having directly committed an incriminated act.

II.1.2. Structuring of the different Ethics Committees

A CERSH is made up of an office, a secretariat and members [2]. The office includes a president and a vice-president who are elected by a simple majority of votes cast by their peers, or by consensus for a period of three (03) years, renewable two (02) times. The operation of the CNERSH is ensured by a technical secretariat placed; under the responsibility of the president.

❖Structuring of the technical secretariat

The CNERSH technical secretariat is coordinated by a Secretary Permanently assisted by four (04) staff including one (01) accountant, one (01) lawyer, one (01) secretary and one (01) technical manager, designated by the Minister in charge of Public Health.

❖ Structuring of the secretariats of a CERSH

The CRERSH and the CIERSH each have a secretariat managed by a secretary, and which is set up by their presidents. The secretariats of the CRERSH and the CIERSH carry out the same missions as the technical secretariat of the CNERSH, namely:

- Manage committee mail

-reception of user information and compliance control of files received;

-the collection, recording and coding of physical ethics clearance request files;

- the registration and distribution of the committee's;

-the reproduction of individual acts and any other service document;

-notification to users of signed documents;

-the creation of electronic files;

-managing the conformity of the form of the administrative file (header, stamps and possibly logo);

-maintain a specific register for submitted clinical trial protocols;

-maintain a general register of submitted, approved research protocols;

-present the committee's activity report and send them periodically to the Minister in charge of Health through the Division of Operational Research in Health.

- prepare the sessions;
- monitor the implementation of the committee's recommendations;
- propose a budgeted action plan as well as the annual schedule of sessions to be submitted to the committee;
- participate in controlling the committee's expenses by co-signing the bank accounts;
- keep the committee's accounts;
- ensure compliance with regulations in the ethical evaluation process, from reception to monitoring of research protocols;
- contribute to the mobilization of resources for the functioning of the committee;
- ensure capacity building of committee members;
- carry out all other missions assigned to it by its supervisory authority,

❖ **Role of the president**

The president ensures the proper functioning of the Ethics Committee.

To this end, he:

- officially represents the committee;
- convenes and chairs the sessions (ordinary and extraordinary) of the committee;

- plans the agenda;
- organizes the monitoring of projects submitted for evaluation and answers questions from researchers relating to the committee's decisions;
- signs all documentation related to meeting reports and ethics clearances;
- organizes public communication of the committee for general ethical questions or concerns relating to the evaluation of research protocols.

At the national level, the president of the CERSH must organize the supervision of the Regional Committees and the Institutional Ethics Committees.

❖ Role of the vice president

The vice president:

- replaces the president in his absence;
- assists the president in his functions;
- carries out all other missions entrusted to him by the president.

❖ Role of CERSH members

In general, members of Research Ethics Committees for Human Health evaluate the research protocols submitted to them on a scientific and ethical level. As part of this assessment, they must ensure that any request submitted for ethical clearance is made within respect for dignity, well-being, security, integrity and human rights and in strict compliance with the procedure for evaluating research protocols.

❖Quality, number, mandate of members of a CERSH

The table below specifies for each type of ethics committee, the quality, the number of members, the number and duration of each mandate [2, 11].

Table 1: composition and duration of mandates according to the type of committee

Types of Committees	Number of members holders	Number of alternate members	Term of office of office members	Number of mandate renewals
CNERSH	20	/ /	03 years	02
CRERSH	07	05	03 years	02
CIERSH	07	05	03 years	02

❖ Gender of members of a CERSH

The female gender must constitute at least 30% of the members in the different ethics committees.

❖Profiles of CERSH members

Each member of CERSH must possess recognized moral integrity. In addition to this, there is a requirement to present one of the profiles as described below [2]. The CNERSH must have the following profiles:

- A doctor with at least 5 years' experience in ethical evaluation;
- An expert in ethical and scientific evaluation of clinical trials with experience of at least 05 years;
- An expert in ethical and scientific evaluation of observational epidemiological studies with experience of at least 05 years;
- An expert in ethical and scientific evaluation of studies in the social sciences with experience of at least 05 years;
- An expert in the methodological and statistical evaluation of research protocols with experience of at least 05 years;

- An expert in health law with experience of at least 05 years;
- A representative of the order of doctors,
- A representative of the order of pharmacists
- A representative of the Order of Surgeons-Dentists;
- A representative of the order of medical and health professionals;
- A representative of the association of traditional practitioners;
- A representative of the ministry in charge of scientific research and innovation;
- An expert in charge of legal affairs from the Ministry of Public Health;
- A representative of the ministry in charge of higher intelligence;
- A representative of the Institute of Medical Research and Medicinal Plant Studies;
- A representative of state medical faculties;
- Two representatives of patient associations;
- A representative of the Civil Society;
- A representative of religious denominations.

The CERSH and CIERSH have the following profiles for full and alternate members:

- Three doctors;
- A specialist in social sciences;
- A member of the community;
- A representative of religious denominations;
- A representative of people with particular skills in basic sciences, pharmaceuticals, ethics/bioethics, biostatistics, research methodology or public health.

The CIERSH must have at least 20% of members not affiliated with the institution.

II.1.3. Independent experts or ad hoc consultants

Each Ethics Committee must have a list of experts who can contribute to the evaluation of specific protocols [2]. Reasons why additional or special expertise may be sought include, but are not limited to, the need to:

- provide specialist expertise for review of specific protocols;
- provide additional statistical, methodological or specific knowledge on potentially vulnerable populations;
- provide additional information on one or more legal, ethical, scientific or socio-cultural aspects before carrying out research;
- review protocols or attend meetings to provide training or provide information on any matter of general interest

An independent expert or ad hoc consultant of an ethics committee is not member of it.

11.1.4. Training for CERSH members

Members of the CERSH, and other people responsible for the evaluation, approval and field monitoring of research projects carried out with foreign participants must imbibe the fundamental principles of ethics, existing legal and regulatory framework, guidelines followed, applicable policies and ethical principles [7]. The training of members of CERSH is initial (newly selected members) and ongoing (former members). This training can be carried out within the framework of cooperation between the Committees and with development partners, but also on other favorable occasions.

❖ Initial training (after installation)

Before officially taking office, newly appointed members selected must benefit from initial training in research ethics in order to strengthen their expertise.

This initial training must cover at least the following points:

- procedures for receiving, evaluating and monitoring research protocols;
- the different aspects of ethical evaluation of protocols;
- the ethical evaluation system of Cameroon;
- organization and operation of an ethics committee;
- local and international texts and guidelines;
- the principal fundamentals of research ethics;
- good clinical practices (good clinical practices taken from the last International Conference on Harmonization (ICH) e6 (r2) [12] for members of the CNERSH committee);
- transparency, security and sharing of personal and biological research data (for CNERSH members),

Training channels can be:

- online courses with or without certificate;
- training seminars;
- academic/university training;
- professional immersion in another ethics committee.

❖ **Continuing education**

It must be organized on the basis of expressed needs or with the aim of extending the skills of committee members in a specific area of ethical evaluation. This can be done according to a specific schedule.

It may relate either to the content of initial training, or to good clinical practices and good laboratory practices, clinical trials relating to drugs, vaccines and medical devices and field trials or any other area of interest.

NB; The CERSH secretariat keeps copies of the curricula vitae and proof of training of all members.

II.1.5. Financing of CERSH

Funding for CERSH activities (ethics evaluation sessions, ethics monitoring, capacity building, functioning of the secretariat, etc.) can come from several sources [2] including:

- state subsidies;
- direct payment of fees of ethical evaluation of research protocols;
- donations and legacies;
- the mobilization of funds, through the response to calls for projects;
- revenue from training and any other resources that could be allocated to it.

However, CERSH must not accept any funding from the following sources:

- investigators and/or sponsors of research protocols;
- institutions or supervisory structures when this funding comes from budget lines not formally dedicated.

II.1.6. Ethics evaluation fees in ordinary and extraordinary sessions

The rates applicable at the CNERSH level. CERSH and CIERSH are set according to the type of protocol to be evaluated, the source of funding and the choice of session as summarized in the table below. These fees are used not only to cover the costs of preparing and holding meetings of the ethics committee for the evaluation of research protocols, but also for monitoring the implementation of research in the field. They are broken down according to the distribution key below:

- 60% represents your research protocol review session fees; donation 10% of this amount must be used for the training of committee members;

- 40% represents monitoring and evaluation costs (MINSANTE Operational Health Research Division and Ethics Committee), of which 10% of this amount must be used for the operation of the ethics committee.

Table 2: Ethics review fees by protocol type, funding source and session type

Protocol type	Source of funding	Session type	Evaluation fees in FCFA
Clinical trials of phase 1 or 2	External financing or Organization	Ordinary	1,000,000
	International	Extraordinary	I 500,000
	Institution Cameroonian	Ordinary	500,000
		Extraordinary	750,000
	Independent researcher/ traditional practitioner	Ordinary	100 000
		Extraordinary	150,000
Clinical trials of phase 3 or 4, Field trials, national surveys (in more than one region), multicenter studies	External financing or organization international	Ordinary	600,000
		Extraordinary	800 000
	Cameroonian institution	Ordinary	400,000
		Extraordinary	600,000
	Student	Ordinary	100,000

		Extraordinary	150,000
	Independent researcher/ traditional practitioner	Ordinary	100,000
		Extraordinary	150,000
		Ordinary	400,000

NB:

- the applicable rates detailed above also apply to research protocols promoted by the Health Programs;
- in the event of renewal of ethical clearance, these costs are reduced by half:
- in case of major amendment, the applicable fees are those provided for in the table above;
- in the event of ongoing review and clarifications requested by the Ethics Committee, additional payments (fees) are not requested from researchers;
- the ethical clearance is valid for the duration of the project communicated by the researcher;

11.2. OPERATION OF CERSH

II.2.1. Setting up a CERSH

Initiator	MINSANTE, Regional Delegation of Public Health (DRSP), Head of the Institution
Intervenor(s)	Scientific and Strategic Advisory Commission of the Health Research (CCSSRS), CNERSH, Head of the structure/Board of Directors

General conditions [2]

- The creation and establishment of an Ethics Committee for Research in Human Health (CERSH) of the structures under MINSANTE are made by an act of the Minister in charge of Public Health, if necessary, by the board of directors of the public establishments of a hospital nature.

- The establishment of a Regional Ethics Committee for Research in Human Health (CRERSH) is initiated by the Regional Delegate in charge of Public Health.
- The establishment of an Institutional Ethics Committee for Human Health Research (CIERSH) is initiated by the head of the institution.
- The act of establishing a CERSH results from the evaluation of the request for establishment submitted by the initiator to the Minister in charge of health.
- The evaluation of request files for recognition by regional and institutional committees is carried out jointly by the Research Division Operational Health (DROS) and the National Ethics Committee for Human Health Research (CNERSH).

❖ **Specific conditions**

The observation of the composition of the National Ethics Committee of the Research for Human Health (CNERSH) falls under the jurisdiction of the Minister in charge of Public Health after advice from the Table 1: composition and duration of mandates according to the type of committee (CCSSRS).

❖ **Description of the procedure**

The steps for setting up a CERSH are as follows:

- call for applications for the recruitment of members; indicating the eligibility conditions, the deadline and the place of submission of files carried out respectively by the Minister of Public Health for the CNERSH; Regional Delegates of Public Health Delegates for CERSH; the head of the hospital or research institution concerned for CIERSH;
- selection of the members of the CERSH and CIERSH by a commission which includes at least one representative of the CNERSH and one

representative of the DROS (see section 11.2.2. Selection procedure for the members of the CERSH);

- holding of the Constitutive Assembly of the Committee with election of the executive office by vote or by consensus with as observers/facilitators at least one representative of the CNERSH and one representative of the DROS;
- transmission of the CERSH and CIERSH observation file to the Minister in charge of Public Health and to the person in charge of the structure for structures under the supervision of the Ministry of Public Health. This file consists of;
    - a request for observation addressed to the Minister in charge of Public Health, if applicable to the board of directors of public hospital establishments;
    - the minutes of the Constitutive Assembly of the CERSH accompanied by the main resolutions, dated and signed by the rapporteur and the chairman of the meeting;
    - the attendance sheet of participants at the Constitutive Assembly of the committee including: first and last names, structure, telephone numbers, e-mail address and address;
    - the complete list of members designated for the creation of the Committee including first and last names; profession, home structure and 1 status within the committee (full or alternate);
    - the Curriculum Vitae of each member;
    - the composition of the executive office (surnames, first names and positions);
    - the domiciliation elements of the committee (headquarters, contacts/address, etc.).

- Installation of said CERSH (see section 11.2,3. Procedure for installing an ethics committee).

❖ Documents supporting the procedure:

- the CERSH's request for observation;
- the minutes of the Constitutive Assembly of the CERSH;
- the attendance sheet of participants at the Constitutive Assembly of the Committee;
- the Curriculum Vitae of each member;
- the domiciliation elements of the Committee.

II.2.2. Selection of CERSH members

Initiator:	Manager in charge of the DRSP structure/Head of the MINSANTE/DRSP/ Responsible
Intervenor(s):	MINSANTE/ DRSP/ Head of institutions, members of selection committee

- **Member selection schedule**

The selection of members is done as soon as the Committee is set up, then one semester before the end of the regulatory mandate of the members, when it concerns a renewal [13],

- **Member Selection Commission**

The manager in charge of the structure wishing to proceed with the selection of members, designates by a memo an independent and competent commission for the review of the application files as follows:

- the Minister of Public Health for the CNERSH;
- the Regional Delegate of Public Health for the CERSH;

- The person in charge of the hospital/institution/research for the CIERSH

❖ **Description of the procedure**

- Calls for applications, indicating eligibility conditions, deadline and location of deposit of files made respectively by the Minister of Public Health of the Public for CNERSH, the Regional Delegates of Public Health for CERSH; the head of the hospital institution or of research concerned for CIERSH.
- closure of registration of application files within thirty (30) days following the call for application.

This file includes:

- a Request for Application addressed to the manager who made the call for applications;
- a resume;
- a cover letter;
- any document proving the candidate's experience;
- a letter of designation signed by a manager of the institution of which he is the representative, for members representing institutions.

• Processing of application files: the different application files are evaluated using a grid allowing the following points to be explored:
    - the eligibility conditions set out above;
    - Training on ethics and research (the estimated duration in hours);
    - contributions to research ethics;
    - experience in protection of research participants;
    - place of residence;
    - gender
    - institutional membership;

- the administrative function;
- sociocultural and religious affiliation.

- Ranking of candidates:

It is done according to the priority criteria which are: the rank occupied after study of the file; gender, institutional affiliation, place of residence, sociocultural and religious affiliation.

- Establishment of the list of members retained by the secretariat.
- Transmission of the list of members retained by the competent authority to the Minister in charge of Health for a decision to establish.
- **Documents supporting the procedure**
    - Call for applications for the selection of members of the CERSH;
    - note of appointment of members of the selection commission;
    - application files;
    - evaluation grid for application files;
    - minutes of evaluation/study of application files;
    - list of selected members.

II.2.3. Installation of CERSH

Initiator;	Manager in charge of the structure (MINSANTE/ DRSP/'Head of the institution)
Intervenor (s):	DROS/DRSP/ÇNERSH/Institution, members newly selected following the call for applications

The establishment of the office and members of an ethics committee is subject to the decision of the Minister in charge of Health establishing the said CERSH [2].

During this installation ceremony of the CERSH, the function of session president is carried out by the Minister of Public Health or his representative.

❖ Description of the procedure

MINSANTE establishes the timetable for the installation of the CERSH, and its office by an official note within at least 30 days before the date intended for installation. This note mentions the place, date and time.

The Permanent Secretary for the CNERSH, the DRSP for the CERSH, and the head of the institution for the CIERSH, are responsible for convening the members and the office of the said committee in writing within a period of at least 21 days before the date of installation. The notice must mention the date, place and time of the meeting, and indicate the questions on the agenda.

The agenda includes the following points:

- verification of the presence of selected members and the elected office;
- welcome message from the local manager;
- presentation of the role of the body in charge of operational research in health;
- presentation of the attributions known as the ethics committee;
- reading of the MINSANTE decision noting the composition of the members of the said committee;
- installation speech of members and office by MSP or his representative;
- congratulatory handshake with office members;
- election of the committee office;
- speech by the president of the newly installed committee;
- delivery of tools and materials to the committee office, if available;
- family photos.

❖Documents supporting the procedure

- decision of the Minister of Health noting the composition of the CERSH;
- Attendance Sheets ;
- Installation minutes ;

- Mission contribution ;
- Pictures

II.2.4 Renewal of mandate for members of CERSH

Initiator: Manager in charge of the structure (MINSANTE/ 'DRSP/Head of the institution)

Intervenor (s): MINSANTE/ DRSP/ Head of the institution, Members Selection Commission

The term of office of members is 03 years [11], renewable twice subject to the approval of the member selection committee of the structure concerned.

The renewal procedure is triggered six months before the end of the current term by the president of the committee. The latter informs the competent authority in writing of the need to renew the members of the committee. Otherwise, the supervisory authority initiates the renewal.

This renewal is carried out under the same conditions as those recommended for the appointment of said members, provided that the member submits his file for renewal of mandate within the deadlines and conditions required by the call for applications.

Members retain their seats until the end of the process of renewing their mandate, or until the designation of a person capable of replacing them if necessary.

❖ **Description of the procedure**

- the competent authority informs the president of the CERSH in writing within a period of not more than six months.
- the competent authority triggers the procedure for setting up the committee (See procedure for setting up the CERSH),

- a technical handover of service is made between the old and new offices in the event of a change.
- the minutes and related reports are transmitted to the competent authority.

- **Documents supporting the procedure**
- information letter from the competent authority addressed to the chairman of the committee for the renewal of committee members;
- call for applications for the selection of CERSH members;
- files for request for renewal of the mandate;
- minutes and related reports

II.2.5. Replacement of a resigning or deceased member

Initiator:	President of the CERSH
Speaker(s):	President of CERSH, Competent Authority (MINSANTE/ DRSP/ Head of institution)

If a member of the CERSH wishes to resign [13], he must notify the president in writing, at least three months before the effective date of his resignation. If an immediate replacement is not required, the selection of the new member will be made at the scheduled end of the mandate.

This replacement method also applies in the event of prolonged absence (six consecutive sessions) or death of a member.

❖ **Description of the procedure**

- the resigning member informs the president of the CERSH in writing in a period of at least three months before the effective date of his resignation; the president of the CERSH acknowledges receipt of the resignation letter;

- the president of the CERSH notifies the competent authority of the resignation of one of its members;
- the competent authority organizes the replacement of said member (See procedure for selection of CERSH members);
- the resigning member is replaced during a committee session by an alternate member whose profile is as close as possible and after deliberation of the committee;
- the minutes and the session report must clearly mention the information relating to the replacement of the said resigning member;
- a notification of replacement is made to the competent authority.

❖ **Documents supporting the procedure**

- letter of resignation from the member;
- notification letter addressed to the competent authority by the president of the CERSH;
- minutes and meeting report mentioning the replacement of the resigning member.

11.2.6. Revocation of a member

Initiator: President of CERSH

Intervenor(s): President of CËRSH, all members, competent authority

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- A member may be dismissed by the committee [13] which expresses itself by a vote of 2/3 of the members present at; an ordinary session for the following reasons:
- prolonged absences from sessions: six (06) consecutive unjustified absences during one (01) year;
- non-respect of confidentiality, repeated verbal and/or physical violence,
- non-declaration of conflict-of-interest situations.

❖ Description of the procedure

- the president of the CERSH included in the agenda of an ordinary session, the deliberation on the subject of the evocation of a member;
- the member concerned must be contacted in writing eight (08) days before the holding of this session to provide explanations for the breach(es) with which he/she is accused;
- the members of the committee express their opinion by vote, the decision of revocation is taken if the quota favorable to the revocation is 2/3 of the members present;
- the president of the CERSH sends a letter informing the competent authority of the CERSH decision, attaching the minutes of the deliberations;
- the competent authority carries out the revocation by a reasoned act and notifies it to the member concerned;
- the competent authority organizes the replacement of said member (See procedure for replacing CERSH members).

❖ Documents supporting the procedure

- Summons addressed to the member accused by the president of the CERSH;
- written justification of the summoned member;
- minutes of deliberations;
- information letter on the revocation of a member, addressed by the president of the CERSH to the competent authority;
- letter of dismissal decision sent by the competent authority to the CERSH member in question.

II.2.7. Selection of Independent Experts and Consultants

- **initiator:** **President of CERSH**
- **Intervenor(s):** **President CERSH, Technical Secretariat. Expert/consultant, competent authority**

If necessary, the CEHSH can call on external experts for the evaluation of a research project requiring specific advice for certain aspects [2,13].

The member(s) may be invited to participate in a session of the committee, but they do not take part in the deliberations of the committee.

- **The Description of the procedure**

- The secretary of CERSH and submits to the president the potential(s) expert(s) from the expert database available at the technical secretariat of the committee, and collects the following information on the candidate, Name, personal and institutional addresses, profile and areas of expertise. The expert must be unaffiliated with the researchers or the institution offering the research protocols.
- the chairman of the committee sends a letter of invitation to the expert(s) whose profile corresponds to the need expressed in the protocol to be evaluated. This letter specifies all aspects of the requested devaluation (e.g.: ethical, scientific, legal, socio-cultural, sociological, anthropological, etc.), as well as the deadlines granted for this evaluation.
- After having acknowledged receipt of the request for expertise, the expert(s) must make a conflict-of-interest declaration in writing and also sign a commitment to respect confidentiality at the technical secretariat of the committee;
- the technical secretariat, on the orders of the president, entrusts to the expert against discharge in a specific register, a copy of the research protocol to be evaluated, as well as an evaluation grid of the protocol containing the ethical aspect(s) and/or scientific(s) that the expert(s) will have to examine;

  Within a deadline defined by the president of the CERSH in his letter of invitation, the expert(s) will send to the technical secretariat of the committee his/her evaluation report(s) against discharge in a specific register.

❖ **Documents supporting the procedure**

- expert list/database;

- letter of invitation to the expert from the President of the CERSH conflicts of interest declaration form;
- confidentiality sheet;
- specific register of experts;
- study protocol;
- protocol evaluation grid;
- protocol evaluation report

II.2.8. Development and validation of the CERSH budget

Initiator: President of the CERSH

Intervenor (s) President, Technical Secretariat, CERSH members, competent authority

The CERSH budget [2, 9, 14-16] results from a validated annual action plan by the office and the members of the committee assisted by representatives of the competent authority.

- **Description of the procedure**

- the secretariat expresses the needs necessary for the smooth running of each activity;
- the CERSH develops an action plan. detailed annual;
- the secretary compiles the expressed needs, prepares the documents relating to the budget validation meeting, in particular the report and the financial statement for year n-1;
- the secretariat and the CERSH office prepare a budget proposal which must include the following lines: sources of revenue, operating costs, protocol evaluation session costs, training costs and monitoring and dissemination costs assessment;
- the president of the CERSH signs the note inviting the validation meeting; an assembly composed of at least one representative of the competent authority, the members of the CERSH and any other person of its choice;
- the secretary presents the budget proposal to be validated during a CERSH session;
- the assembly amends, validates and adopts the budget proposal;
- the validated budget is shared with the competent authority.

- **Documents supporting the procedure**
  - action plan with proposal for the preliminary draft budget;
  - minutes of the budget validation meeting;
  - report of the budget validation meeting;
  - the budget validated by the general assembly.

II.2.9. Revenue collection of CERSH

Initiator:	**President of the CERSH**
Speaker(s):	**President CERSH, Technical Secretariat, accounting, competent authority**

Revenue collection is done in compliance with regulations in force in financial management.

These revenues may come from the direct payment of assessment fees for protocols [16], state subsidies, donations and legacies or fees institutional (overheads).

❖ Description of the procedure

- the CERSH makes public its bank identity statement and the protocol fee schedule;
- the researcher pays to the bank where the account is domiciled against a receipt, the amount corresponding to the protocol submitted;
- the payment receipt is attached to the protocol submission file;
- confirmation of payments (account history) is presented to the CERSH during each session;
- the payment is recorded in the CERSH accounting documents,

❖Documents supporting the procedure

- Bank details;
- payment slip;
- accounting documents (journal book, cash flow report, bank account history)

- **Description of the procedure for grants, donations and legacies from partners**

  - The grant, donation and bequest documents are presented to the committee by a representative of the lessor or donor
  - the committee takes note of and validates the financing;
  - the contract is signed with the lessor following the clauses validated by both parties;
  - the funds are transferred to the CERSH account;
  - confirmation of the transfer is presented to the committee during a session;
  - Each payment is recorded in the CERSH accounting documents.

- **Support documents for your procedure**

- documents justifying the financing;
- committee meeting report including validation of funding;
- contract signed between the lessor and the CERSH;
- accounting documents (journal book, cash flow report, bank account history).

- **Description of the procedure for state subsidies**

The allocation and recovery of this source of financing is done according to the provisions of the finance law in force.

- **Support documents for the evaluation grant procedure**

- budgetary accreditation vouchers;
- Purchase Orders;
- commitment vouchers.

Expenditure management of CERSH

Initiator:	**President of the CERSH**
Speaker(s):	**Chairman Secretary and Accountant of the Committee, State Financial Control**

All funds generated and managed by ethics committees constitute public funds and are therefore subject to management and public accounting rules [14].

- Description of the procedure
- the president of the CERSH is the main authorizing officer of expenditure;
- financial operations in the accounts are subject to the requirement of a triple signature of the Chairman of the Committee, the Permanent Secretary (CNERSH) or secretary (CRERSH or CIERSH) and the accountant or any other member duly designated by the President, where applicable;
- financial control verifies and validates the budgetary alignment of the expenditure;
- the accountant checks the conformity of the expense report;
- the accountant disburses and liquidates all expenses ordered by the president;
- the expense incurred is entered in the expense journal book;
- the documents relating to each expenditure executed are kept by the accountant who copies them to the president and the technical secretariat of the committee.

- **Documents supporting the procedure**
- expenditure authorization;
- bundle of expenses;
- accounting journal book.

II.2.11. Accountability of CERSH

Initiator:	President of CERSH
Intervenor(s):	Chairman, secretary and accountant of the committee, financial control of the State

At the end of each budgetary year, accountability is an essential step in the management process.

- **Description of the procedure**
- the accountant or any other member duly designated by the Chairman of the Committee
- Ethics establishes all statements relating to the situation of the bank account and all resources allocated to the CERSH and transmits them to the committee secretariat;

- the permanent secretary or secretary prepares and presents the financial report for the past financial year as well as a report on the state of the Committee's assets [9];
- the members of the committee validate all the financial documents during a session at which the CERSH supervisory authority is represented;
- the president of the CERSH can order an annual financial audit carried out by an independent expert.

- **Documents supporting the procedure**
- financial reports;
- employment accounts:
- assessment of the state of the heritage;
- statement of bank accounts;
- bundle of expenses;
- financial audit report if applicable.

II.2.12. Conducting sessions/meetings of CERSH

Initiator:	President of the CERSH
Speaker(s):	CERSH members, permanent secretary/secretary

The committee meets in ordinary and extraordinary sessions.

- **The ordinary session**

Ordinary sessions of the committee are held monthly. The annual calendar is published in advance no later than October 30 of each year. In the event of a modification of the initial schedule, the chairman of the committee will make public the new date(s) no later than one month before the modified date(s).

The sessions are held on the principle of respecting the following aspects: speaking time, kindness and confidentiality.

Independent experts may be invited to sessions and/or send their comments on points or subjects on which they are requested by the committee.

Principal Investigators, sponsors or researchers may be invited during the sessions to provide clarifications on their projects if necessary.

- **Description of the procedure for the ordinary session**

-the chairman of the committee sends a notice to the members which includes the place, date, time and agenda of the session;

-convocations are sent by letter, fax or any other means leaving a written trace, by the President at least fifteen (15) days before the date scheduled for the session;

-the president checks the quorum required for the holding of a session which is 2/3 members present;

- The president presents the agenda, which is validated as is or modified by the members;

-the participants in the session each introduce themselves individually:

-each evaluator signs the conflicts of interest declaration and the confidentiality form;

-the secretary presents the list of protocols to be evaluated;

-the evaluators each present their evaluation report;

-the members of the committee make decisions by consensus or vote (see decision-making procedure Pxx);

-the permanent secretary draws up and presents a session report at the end of the work;

-the members approve and validate the minutes of the session.

Support documents for the procedure

- letter of invitation from CERSH members;
- 85 session agenda;
- Session attendance list;
- minutes of session,

- **Description of the procedure for the extraordinary session**

It is a session whose necessity is dictated by a situation of documented health emergency and/or administrative emergency [13]. Health emergencies are defined as the appearance or imminent threat of a disease or emergency. state of health which poses a significant risk of occurrence of numerous incidents of destruction of installations. human, permanent or long-term disability. They can have causes [17], including (separately or in combination):

- epidemics/pandemics of infectious diseases such as Ebola virus disease, measles, Zika virus disease or Lassa fever, etc;
- natural disasters, for example earthquakes, tsunamis, floods or cyclones;
- man-made disasters, such as large-scale industrial accidents, conflicts and massive population displacements.

❖ **Description of the procedure**

- the principal investigator or health authority requests a session to be held extraordinary to the president of the CERSH, and submits the submission file for the research protocol (see related procedure);
- the President of the CERSH convenes the extraordinary session no later than five (05) days after recording the request from the principal investigator, informs and immediately assigns the protocols to the evaluator members;
- the session involves a maximum of one third (1/3) of the committee members;
- the meeting to evaluate the submitted protocol is held no later than ten (10) days after registration of the ethical clearance request file;
- the committee has a maximum of five (05) days after the session to make the decision to the principal investigator.

❖ **Documents supporting the procedure**

- request to hold an extraordinary session addressed to the president of the CERSH by the health authority or principal investigator;
- convening of the extraordinary session;
- session agenda;
- session attendance sheet;
- session minutes.

II.2.13 Participation of the principal investigator to a session of CERSH

Initiator:	Committee Chair/Principal Investigator
Speaker(s):	President of CERSH, Technical Secretariat. Investigator principal/co-investigator(s), members of the CERSH

When the CERSH needs additional information on a file to make a decision, it can invite the principal investigator to attend a session [13]. However, its

presence is not obligatory, but desirable. The principal investigator can also request their presence at the evaluation session of their protocol.

❖ **Description of the procedure**

- the committee chair notifies the principal investigator of the date of the session 14 days before the scheduled date for an ordinary session, and five days for an extraordinary session (the co-investigator(s) can also appear in place of the principal investigator);
- the principal investigator may be invited to give a 15-to-20-minute presentation of their protocol, before responding to questions, concerns and suggestions from committee members;
- At the end of the discussions, the principal investigator will be invited to leave the room during deliberations on its research protocol;
- the members of the CERSH deliberate and make decisions by vote or by consensus;
- the principal investigator will be notified of the committee's decision by the usual procedure (. PageXXX).

❖ **Support documents for the procedure**

- letter of invitation to the session addressed to the principal investigator by the president of CERSH;
- Receipt acknowledgment from the principal investigator;
- session agenda;
- attendance record;
- session report.

II.2.14. Management of conflicts of interest during ethical evaluations of research protocols

Initiator:	**President, CERSH members, CERSH Technical Secretariat**
Speaker (s):	**President, technical secretariat, CERSH members**

A conflict of interest arises when a tension exists between the obligations or responsibilities of a person participating in an evaluation process and their personal, professional, institutional or financial interests [7]. A lack of impartiality may be feared when a person's judgment or decision is likely to be influenced by competing interests.

For example, there may be a real, potential or apparent conflict of interest when the member participating in the evaluation process is in one of the following situations:

- the member may receive an advantage or suffer a personal disadvantage, of a professional, institutional, financial or other nature, resulting from the conduct of the research carried out;
- a member of the immediate family (spouse, child or parents), extended family, a friend or a person in whom the assessor demonstrates a particular interest, may receive a benefit or suffer a personal disadvantage, of order professional, institutional, financial or other, resulting from the conduct of the research carried out:
- the member maintains a recent or significant professional relationship, in such a way as to create a favorable or unfavorable prejudice (conflictual relationship) with a person who may receive an advantage or suffer a personal disadvantage of a professional, institutional, financial or other nature, resulting from the course of the research carried out;
- the member has an institutional or organizational link, or even disciplinary affiliation with the principal investigator, a co-investigator or a person involved in the preparation or implementation of the research protocol;
- the member is involved in the preparation or implementation of the research protocol;
- the chair of the committee or one of the members submits an application as principal investigator, co-investigator or collaborator;
- any other situation which suggests a real, potential or apparent conflict of interest.

Evaluators are required to declare any situation of real, potential or apparent conflict of interest to the chair of the committee. The chair of the committee is responsible for determining whether the conflict of interest expressed is likely to influence the independence and integrity of the evaluator.

In the event of a conflict of interest of the committee president, the vice-president or the oldest member without a conflict of interest chairs the session.

CERSH members involved in the preparation or implementation of a research protocol cannot participate in monitoring the implementation of this research in the field.

❖ Description of the procedure

Once ethical clearance requests have been submitted:

- the permanent secretary/secretary transmits the list of protocols submitted, the names of the principal investigator and co-investigators to the president of CERSH;
- the chair of the committee verifies with each potential evaluator the presence of a real, potential or apparent conflict of interest before assigning protocols for evaluation;
- evaluators who are assigned a research protocol sign the conflict-of-interest declaration form;
- in the event of a declared conflict of interest, the committee chair identifies new reviewers without a conflict of interest for the applications in question;
- at the start of the committee session, the president asks the evaluators to announce to the other members of the evaluation committee whether they are in a real, apparent, potential, or not conflict of interest; and reminds members with conflicts of interest that they must withdraw from the room;
- In the event of denunciation of a conflict of interest for an evaluator, the president suspends the debates and carries out the verification; if the conflict of interest is proven, the evaluation report pre-established by the evaluator in question is purely and simply canceled,

❖ Documents supporting the procedure

Conflict of interest declaration form.

II.2.15. Drafting minutes of CERSH sessions

Initiator:	Permanent Secretary/Secretary of CERSH
Speaker (s):	Permanent Secretary/Secretary, President of the CERSH, members of the CERSH

The decisions of the CERSH taken in sessions must be the subject of minutes in order to document the deliberations [2, 13].

❖Description of the procedure

-the minutes of sessions are drawn up by the Permanent Secretary/Secretary of the CERSH, and must contain at least the following information:

- The date, time of opening and closing of the session;

- The names of CERSH members present and absent and, if applicable, those of external experts and guests;
- The list of protocols evaluated during the session with version references;
- The names of the principal investigators. evaluated research projects;
- The origin of funding for research projects;
- The results of the discussions, in particular the number of protocols approved, the number of refusals, and the number of requests approved subject to reservation,
- the minutes must be signed by each member of the CERSH present at the session;
- the permanent secretary/secretary archives the validated minutes;
- the final minutes of the sessions are sent to all members of the CERSH and archived by the technical secretariat with all other documents relating to the protocols evaluated.

❖ Documents supporting the procedure

Validated session minutes.

II.2.16. Writing the CERSH session report

Initiator:	Permanent Secretary/CERSH Secretary
Speaker(s):	Permanent Secretary/Secretary. President of CERSH, members of CERSH

Reading the report should make it possible to follow the reasoning underlying the decision and, where applicable, the dilemmas that the Committee was confronted with or the divergent positions.

❖Description of the procedure

- session reports prepared by the Permanent Secretary/Secretary of the CERSH [9] must contain at least the following information:
- the date, opening and closing time of the session;
- the names of CERSH members present and absent and, if applicable, those of external experts and guests;
- the list of protocols evaluated during the session with version references;
- the names of the principal investigators of the research projects;
- the origin of funding for research projects;

- the summaries of the projects, in particular the objectives, the profile of the anticipated research participants, the planned duration, the nature of the procedures;
- the summary of the discussions, in particular the opinions expressed, the controversial points, the conclusion reached by the CERSH which was clearly justified on an ethical and scientific level, as well as the disagreements with the justifications;
- the outcome of the discussions, including:
- the number of protocols approved;
- the number of refusals and their motivations;
- the number of applications approved subject to reservation and the description of all the requirements posed by the CERSH to the principal investigator;
- in cases of decision-making by voting, the number of votes in favor, against and abstention;
- any situation of real, apparent or potential conflict of interest and the withdrawal of a member during deliberations.
- the report must be sent to each member of the CERSH who participated in the session electronically.

❖ Documents supporting the procedure

- session minutes;
- session report.

II.3. MONITORING AND EVALUATION OF THE FUNCTIONALITY OF CERSH

Initiator: Competent regulatory authority

Speaker(s): Managers of the competent institutions: MINSANTE. CNERSH, DRSP. Research Institutions

The Division of Operational Research in Health (DROS), the Regional Delegation of Public Health (DRSP) and the head of the institution respectively monitor the functionality of the CNERSH, the central CERSH and the central CIERSH.

The CERSH will be supervised on indicators relating to:

- the constitution of ethics committees (the office, the members, the Technical Secretariat);
- conformity of the composition of members (profile, gender aspect, number)

- member training;
- respecting the frequency of research protocol evaluation sessions;
- compliance with protocol submission and evaluation procedures;
- monitoring research projects that have obtained ethical clearance and an AAR;
- the renewal of Committee members;
- the existence of office and computer equipment;
- the independence of the committee vis-à-vis the institution which hosts it, the researchers, promoter and sponsor;
- the sources of financing of the committee;
- management of conflicts of interest;
- the partnership;
- and the production of activity reports.

Furthermore, the CNERSH supervises the CERSH and CIERSH [2]. These structures will be supervised on indicators relating to:

- the constitution of ethics committees (the office, the members, the Technical Secretariat);
- conformity of the composition of members (profile, gender aspect, number)
- member training;
- respecting the frequency of research protocol evaluation sessions;
- compliance with protocol submission and evaluation procedures;
- monitoring research projects that have obtained ethical clearance and an AAR;
- the renewal of committee members;
- the existence of office and computer equipment;
- the independence of the committee vis-à-vis the institution which hosts it, the researchers, promoter and sponsor;
- the sources of funding for the committee;
- management of conflicts of interest;
- partnership and production of activity reports;
- the quality of the ethical evaluation of research protocols.

The monitoring and evaluation grid for human health research ethics committees (CERSH) is presented in the appendix to this guide.

❖ **Description of the procedure**

Each ethics committee will send an annual report regarding it to:

- Minister in charge of health for the CNERSH;
- Regional delegate for public health and CNERSH for CRESH;
- Responsible for the institution for CIERSH.

The annual report of an ethics committee must contain at least the following information:

- the list of members, specifying their skills (profession and professional affiliations) as well as their role within the CERSH;
- the list of protocols submitted, those having received approval and those rejected with information such as the name of the sponsor, the field of research, the sponsor, the target, the area of implementation, and the period of the study;
- the number of conflict-of-interest declarations;
- the number of sessions held during the year, and the minutes of the sessions which must include:
- the date, opening and closing time of the session;
- the names of CERSH members present and absent and, if applicable, those of external experts and guests;
- the list of protocols evaluated during the session with version references;
- The names of the principal investigators of the research projects evaluated;
- the origin of funding for research projects;
- the outcome of the discussions, including the number of protocols approved, the number of refusals, and the number of requests approved with reservations.
- ethical monitoring activities of research projects carried out;
- the frequency of activities relating to continuing education in ethics;
- the proportion of members having followed training in research ethics

The DROS must actively supervise the CERSH, with the aim of verifying the state of the infrastructure, office equipment and consumables, the committee's findings documents, the minutes of the sessions, and other documents deemed relevant by the supervisory team.

The supervision of ethics committees can be formative and/or evaluative.

❖Support documents:

- CERSH annual report;
- notes from supervision missions;
- supervision grid
- reports of supervision,

Chapter III: Ethical and Administrative Evaluation of Research Protocols for Human Health

All research conducted with human participants is evaluated and approved by a CERSH and MINSANTE. The decision-making and monitoring bodies for the conduct of any human health research project in Cameroon are: the competent Ethics Committee and the DROS.

According to article 34 of the law of April 27, 2022, any research protocol must, before its implementation, obtain an Ethical Clearance issued by the competent ethics committee and an Administrative Research Authorization (AAR) issued by MINSANTE [1]. DROS only examines files that have previously obtained ethical clearance.

No intervention or interaction may be undertaken with human participants in research, including recruitment, without an evaluation and research protocol approval, consent documents and recruitment materials.

The diagram below represents the process of processing files for ethical clearance and Administrative Research Authorization (AAR), and monitoring the implementation of a health research project in Cameroon.

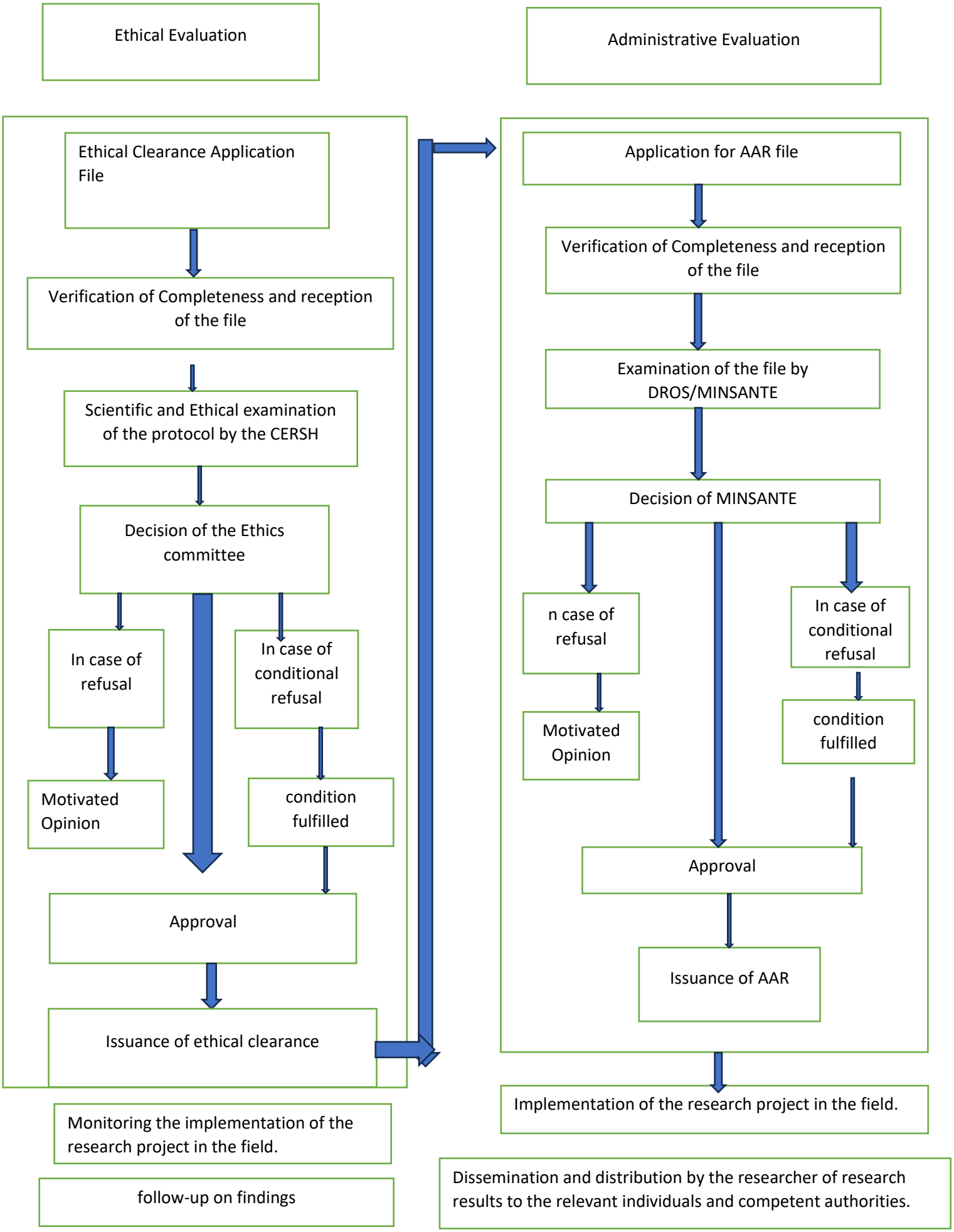

Figure 1: Process of processing applications for ethical and administrative clearance, as well as monitoring the implementation of a research project in human health in Cameroon

III.1. ETHICS ASSESSMENT PROCEDURES

III. 1.1. Submission of research protocols

Initiator:	**Any principal investigator wishing to conduct a human health research project in Cameroon**
Speaker(s):	**CERSH**

- **Condition for acceptability of protocols**

Any research protocol for human health is eligible for ethical evaluation of the CERSH if the object of the study relates to the human person and/or their interactions with their environment, in particular biological, sociodemographic data and life stories [1]. Research relating to the environment, agriculture, water or food with effects on biology, physiology or human health is therefore also considered to be research for human health.

A request for ethical evaluation of a research project involving a human person must be submitted by a researcher/principal investigator, responsible for the ethical and scientific conduct of the research project. The principal investigator must have a physical address in Cameroon, and where applicable, have a co-investigator (local principal investigator) with a physical address in Cameroon.

The principal investigator must be "a person who has acquired appropriate education, training and qualifications in ethics and science, including physicians of all specialties, pharmacists and oral physicians [1].

The CERSH can only receive protocols falling within their competence. Submission of a complete file must be done at least twenty (20) days before the date of the session in question.

- **Description of the procedure**

For the initial submission of the ethical clearance request file for a research project to the CERSH:

- The principal investigator must submit all the required documentation in physical and digital versions to the CERSH secretariat. This file includes the following documents [18J:
- A request for ethical clearance addressed to the chair of the committee and signed by the principal investigator. When the principal investigator does not have an institutional and/or physical address in Cameroon, the application is co-signed by the principal investigator and the investigator residing in Cameroon;

- A paginated research protocol, written either in French or English, signed by all investigators and: including P postal address, email, telephone contact and institutional affiliation of the investigators. This protocol is presented according to the plan below:
- the cover page including: title of the protocol, protocol identification number, version number and date, name and address of the promoter and/or sponsor, name and detailed contact details of the principal investigator (institutional affiliation, postal contact, telephone and fax numbers), signature of the investigators;
- The summary indicating the pages, titles and subtitles detailed in the protocol;
- The list of abbreviations ;
- The glossary presented in alphabetical order; o The list and contacts of investigators;
- Structured summaries in French and English (justification - objectives-methodology-expected results-keywords);
- The introduction (context and justification);
- Research questions and hypotheses where applicable;
- The objectives;
- The literature review;
- The methodology including the type of study, the site, the period and the study population, the selection criteria, the sampling procedure and the estimation of the sample size, the definition of the key variables of the study, data collection tools and techniques, study procedures, data analysis plan, ethical considerations and results dissemination plan.

NB: In the case of clinical trials, specify the phase of the trial, the method of randomization, the characteristics of the interventions received by the intervention group and the control group, the method of measuring the main effect sought and its estimation. implementation timeline;

o the detailed local currency budget of the project which indicates the sources of financing, name and contact details of the promoter (sponsor);

- bibliographic references in accordance with a reference standard;
- informed consent documents (information notice, consent form and possibly assent form and parental authorization) written in English and French and, if necessary, in any other language making them accessible to the study community;
- data collection tools in French and English;
- the curricula vitae of the principal investigator and co-investigators of the research team up to date, signed and dated, indicating at least the training received, their affiliations, the research projects in which they

have been involved, the list of publications showing their experience in implementing the submitted research protocol;
- the different ethical clearances obtained where applicable;
- the Principal investigator brochure (clinical trials);
- the material transfer agreement if applicable;
- the data transfer agreement if applicable
- a letter of commitment on the honor of returning the research results accessible to the research population;
- an agreement in principle from the manager of the sites where the research will be carried out.

-the secretariat verifies the completeness of the file upon receipt and the receipt of payment of fees to the CERSH account;

-the secretariat issues a deposit receipt to the bidder in the form of a file completeness checklist signed and dated by the secretariat.

❖**Support documents for the procedure;**

- complete research protocol submission file to CERSH;
- checklist for checking the completeness of the submission file;
- deposit receipt to the bidder,

III.1.2. Submission of protocols for special cases

For particular cases of interventional studies such as Clinical Trials with intervention products (such as drugs, Improved Traditional Medicines (ATMs), medical devices and medical techniques), in addition to the general elements mentioned above, other elements specific must be included in the protocol submission file to the National Ethics Committee for Human Health Research.

- **Supplement to the protocol submission file for Clinical Trials with intervention products**

For clinical trials, the research protocol submission file must be completed with the elements described below:

- the promoter's commitment to ensuring the participant's access to the product who has not yet obtained a Marketing Authorization (MA) if the latter proves effective; this over a period of at least 3 years;
- a special exemption for the importation of experimental products that do not yet have a marketing authorization;
- a valid insurance policy for researchers and all research participants covering damages related to the administration of the product and the implementation of the procedures and errors of the methodology;
- the “investigator’s brochure” concerning the product subject to the experiment and/or its information leaflet if the product has a marketing authorization.

Depending on the experimentation phase, the investigator's brochure must contain the information specified in Table 3.

Table 3: Information from the investigator's brochure according to the phases of the clinical trial

Phases EC	Information to be found in the investigator’s brochure
Phase 1 (préclinique)	- The active ingredient - The list of excipients The different dosages tested with their pharmacokinetic and pharmacodynamics (acute and chronic toxicity on noble organs, teratogenicity, lethal dose and tolerated dose); the in vitro and in vivo effects on the targeted germs if it is an anti-infective - The number and type of animals used for the experiment - Indications and contra indications - Drug interactions - Storage conditions - Reference for possible publications 1
Phase 2	Information from phase l supplemented by: pharmacokinetic and pharmacodynamic data in humans (frequency and severity of effects on noble organs, the different doses tested, list of serious and minor adverse events detected as well as analysis of their attributability; the number, sex and age of human subjects (participants); the summary of the methodology and procedures of phase 1; Indications and contraindications in men; - Drug interactions ; - Storage conditions ; References for possible publications. Information from phases I and 2 supplemented by: the number of participants. ; the administration protocols of phase 2 and the estimation of the indicators of their effectiveness *: the list of serious and minor adverse events detected as well as the analysis of their attributability; - Summary of the methodology and procedures of phase 2 - Indications and contra-indications; - Drug interactions; - Storage conditions; References for possible publications
Phase 3	The physical, chemical properties and formulation of the product (the

Phase 4(après obtention d'une AMM)	Indications and contraindications: drug interactions and storage conditions) - The number of individuals in whom the product was used in preclinical phase 1, 2 and 3 and the probability of effectiveness - Serious adverse reactions (SAE) already recorded in all previous phases - Phase 3 administration protocols and estimation of their effectiveness indicators - The list of serious and minor adverse events detected as well as the analysis of their attributability - Summary of the methodology and procedures of phase 3 - References for possible publications

❖ Supplement to the protocol submission file for Improved Traditional Medicines (ATM)

The Improved Traditional Medicine (ATM) is any medicine/product developed and developed by a traditional practitioner or a researcher based on knowledge or information from traditional therapeutic heritage [19].

Any request for evaluation of clinical research protocols with MTA must be completed with the following documents i

- a letter of motivation to carry out this research duly signed by the promoter;
- ethno-pharmacological evidence;
- P identification of metabolites (MITA categories 3 and 4);
- the results of efficacy and toxicity studies (MTA categories 3 and 4);
- the galenic formulation;
- the description of active ingredients and pure molecules (MTA categories 3 and 4);
- the description of the structure-activity relationship (process by which a chemical structure is correlated with a well-determined effect such as biological activity or chemical reactivity) (MTA categories 3 and 4);
- the result of the non-toxicity assessment carried out by the DPML and LANACQME (MTA categories 3 and 4).

NB: According to the category indicated in the table below, the file must be completed by the corresponding elements.

Table 4Research protocol submission file supplements according to category identification, evidence of safety and effectiveness of MTA.

Categories [19]	**Identification elements**	**Methods for Proving Safety and Effectiveness**	**Completing the file**
Category 1 Medications	Medicine prepared by the traditional practitioner health for a patient in a way externally according to the methods traditional production and standardization, and which meets the following characteristics i)its safety and effectiveness are guaranteed by long experience of its use (three generations); ii) the raw materials are well known to the traditional practitioner and can be fresh or dry; iii) its conservation is generally short-lived.	Traditional use with extensive experience of its usage across more than 3 generations.	Report from ethno-pharmacological studies on the field
Medicine of category 2	Medicine from traditional therapeutic heritage commonly used in the community which meets the following characteristics: i) it is prepared in advance, packaged with a batch number; ii) the raw materials used in its composition are very well known to the population; iii) its production is carried out using	The ethnomedical evidence of a long history of use or through open clinical trials if deemed necessary by the competent authority.	A report of the field ethno-pharmacological study. or A report on the results of studies using retrospective treatment-outcome methods when the doses according to

	methods that guarantee its stability and standardization; iv) its production is industrial or semi-industrial; v) its safety and effectiveness are guaranteed by ethnomedical evidence from long experience of use or by open clinical trials if this is deemed necessary by the competent authority; vi) the active ingredients which compose it are raw materials; vii) the main chemical groups of the raw materials are known; viii) its shelf life is determined by stability tests.		indications are already known and practiced in the population (reverse pharmacology).
Medicine of category 3	Medicine resulting from scientific research whose active ingredients are standardized extracts and which meets the following characteristics: i) it is prepared in advance, packaged with a batch number; ii) its production is semi-industrial or industrial; iii) the duration of its conservation is fixed through stability trials iv)The active principles are standardized extracts v) It takes into consideration the biological properties of raw materials, new therapeutic indications, galenic formulation with dosage specifications,	preclinical and clinical studies conducted following standard protocols	Results of preclinical studies (botanical/ethnobotanical, pharmacological and toxicological study) and treatment results when the doses according to indications are already known and practiced in the population (reverse pharmacology).

	and knowledge of active biological molecules vi) It is standardized and produced according to good manufacturing practices standards vii)Its efficacy and safety are proven through preclinical and clinical trials conducted following standard protocols.		
Medicine of category 4	Medicine resulting from scientific research whose active ingredients are purified molecules and which meets the following characteristics: i) it is prepared in advance, packaged with a batch number: ii) its production is semi-industrial or industrial: iii) its shelf life is determined by stability tests; iv) the active ingredients are purified molecules: v) it takes into consideration the biological properties of the raw materials, new therapeutic indications, a galenic formulation with a dosage specification, knowledge of biologically active molecules; vi) it is standardized and produced according to good manufacturing practice standard:	Trials preclinical and clinical studies conducted following standard protocols	Results of preclinical studies (botanical/ethnobotanic, pharmacological and toxicological study) and Clinical trial / Retrospective method of treatment-result when the doses according to the indications are already known and practiced in the population (reverse pharmacology)

vii) its effectiveness and safety are proven by preclinical and clinical trials conducted following standard protocols

❖ **Supplement to the protocol submission file for projects involving transfer of biological materials/data between teams of investigators**

The file must be completed by the material transfer agreement and/or the data sharing agreement [1].

- The Material Transfer Agreement includes the following information:
- the nature, quantity and frequency of sample transfer;
- the description of the samples in terms of source, volume, method of conservation and transport;
- the name and address of the sender and his institution;
- the name and address of the recipient and their institution;
- the method of destruction and conditions of processing of derived products;
- the conditions for sharing with other laboratories;
- the law applicable to sample security;

NB: the material transfer agreement must be signed by those responsible for shipping and sample security,

- The Data Sharing agreement includes the following information:
  - the list of data collection tools;
  - the databases generated and their location;
  - people with access to these bases;
  - itinerary and data security mechanisms;
  - people who can modify the bases;
  - the retention period of databases;
  - deadlines and conditions for publishing databases
  - the list of expected publications and authors.

III.1.3. Assignment of research protocols to members and independent experts for evaluation

Initiator: Permanent Secretary/Technical Secretary

Speaker(s); Technical secretariat, members of the CERSH

The technical secretariat of an ethics committee assigns the protocols to members or independent experts based on their expertise. The minimum number of reviewers for a protocol is three (03). A proposal for the assignment of the protocol examined is then made by the secretariat and previously submitted for validation by the President at least two weeks before a session is held. The protocols must be assigned to the evaluator members fourteen (14) days before the session.

❖ **Description of the procedure**

- The technical secretariat compiles the newly developed research protocols received.
- Before the session, the electronic version of the ethical clearance request file is shared by the Technical Secretariat to at least three evaluators. The latter can be committee members or external evaluators chosen according to their area of expertise. These external evaluators must be part of a list approved by the committee.
- The declaration of conflicts of interest of evaluators in relation to the protocol to be evaluated is made immediately after receipt of the protocol to be evaluated,
- The technical secretariat then distributes the protocols to the assigned members and notes the elements below in its register:
  - Date of receipt of the protocol;
  - Title of the protocol;
  - Name of principal investigator;
  - Names of members assigned to the evaluation;
  - Date of distribution of protocols to the members concerned.

❖ **Documents supporting the procedure**

- List of protocols submitted for ethical evaluation before the session date
- Protocol allocation sheet for evaluator members
- Technical secretariat register

III. 1.4. Ethical assessment criteria

Initiator: President of the CERSH

Speaker(s): Technical secretariat, members of the CERSH; invited investigators, experts and external evaluators.

The objective of this procedure is to describe the ethical evaluation criteria of the submitted protocol. This evaluation is done in two stages, including an

initial one before the session and another during the session. It must take into account the elements following [7, 12, 20]:

- social value;
- scientific validity;
- analysis of the risk-benefit ratio;
- fair selection of participants;
- the informed consent process;
- respect for confidentiality;
- collaborative partnership to improve access to research results
- respect for human dignity;
- respect for social and cultural norms on an individual and community level;
- the researcher's conflict of interest situation;
- results restitution plan.

❖ **Description of the procedure**

After allocation of research protocols to be evaluated:

- evaluators must send their comments to the technical secretariat in physical form or by email no later than 24 hours before the session; these reports must comply with the committee's ethical evaluation grid;
- the permanent/technical secretary must ensure that at least two out of three evaluation reports have been received for each protocol; otherwise, he urgently reassigns this protocol to another evaluator present at the session:
- the evaluation reports are presented by each evaluator or by the permanent/technical secretary during the session;
- opinions on each protocol are discussed under the moderation of the session chairman;
- the decision is adopted by consensus: otherwise, we proceed by vote and the reservations are recorded in the minutes;
- the minutes of the session must be validated and signed by the members present.

❖ Documents supporting the procedure

- processing sheet for protocol submission files
- physical and digital ethical evaluation grid of the protocol
- evaluator ethics evaluation reports
- session attendance sheet
- minutes of the session

- elements and criteria for ethical evaluation of research protocols (see grid in appendix)

a) social value

It refers to the relevance and level of priorities of the research theme for a community, the direct and indirect beneficiaries in accordance with those defined by the ministry in charge of public health.

The assessors check whether:

- the theme is relevant to population health research objectives;
- the project is in compliance with national and/or global priority health needs;
- the project has a direct and/or indirect benefit for the populations;
- the plan for disseminating the results involves the participants, the community, local and national health authorities, partners working in the health field concerned and the scientific community.

b) Scientific validity

- Generally speaking, assessors must check whether;
- the research question arises from its justification;
- the hypotheses to answer the research questions;
- the objectives allow the research questions to be answered;
- the proposed study design and the participant selection method are adapted to achieve the set objectives ;
- the selection of participants, the estimation of the sample size and the measurement methods proposed help to minimize selection bias, information bias, sampling fluctuations and confounding factors;
- the study process arises from the objectives and hypotheses;
- the method reflects the characteristics of the subjects under study;
- the analysis plan correctly estimates the indicators of the specific objectives set;
- the collection tools make it possible to achieve the objectives;
- each objective has the variables expected in the collection tool;
- the principal investigator has the skills and experience required to conduct the research;
- the research team is competent to meet the research objectives.

- If it is a clinical trial, check if:
- the proposed randomization method allows for a balanced distribution of study participants

- P intervention, control and measurements of the variables of interest are clear and precise;
- the means planned for their future dissemination;
- upcoming publications, if applicable.

When the project duration is less than one year, the researcher must produce a final report containing information similar to the annual report of a research project.

IV.1.2. Active continuous monitoring - monitoring of the modalities of participation in the research

Initiator:	CERSH and MINSANTE
Speaker(s):	CERSH, DROS, research team

Monitoring the terms of participation in research [12, 13] consists of capturing any information concerning the effectiveness of the arrangements to which the principal investigator has committed, namely:

- information/awareness of the participant;
- the collection without constraint of his consent;
- participation in research without constraints;
- respect for the dignity of the participant and the confidentiality of participation in research;
- the progress of the research in accordance with the announced timetable.

The CERSH and the MINSANTE have the power to exercise other means of monitoring concerning the projects; research for which they have given their approval. These means can be

observing the process of obtaining free and informed consent;

b) Annual Report

The principal investigator must submit to the ethics committee and MINSANTE an annual report containing the following information:

- the date of the actual start and planned end of the research activity;
- the progress and progress of the work;
- total recruitment and enlistment time;
- the number of subjects initially approached and the number of those who agreed to participate in the project;

- the number of subjects who dropped out along the way, and their reasons if known;
- problems, adverse events or therapeutic incidents other than serious that occurred during the year, with summary explanations relating to them and a description of the means put in place to help the research subjects;
- assessment of the current risk, potential benefits and overall risk/benefit ratio of the study;
- possible partial results;
- the status of the trial (still in progress or not);
- any other point previously raised during the ethical and administrative evaluation.

c) Notification of the end of the project

The principal investigator will notify the ethics committee and MINSANTE of the end of the project, and provide the following information:

- The obtained results ;

In the case of clinical trials, the report must contain the following information:

- the type and clinical characteristics of the event;
- the dates of occurrence, notification and investigation of the event;
- description of the event investigation team;
- the causal relationship between the event and the intervention;
- the number of subjects affected by this event;
- the description of the management and the clinical evolution of the cases
- the repercussions of this event on the continuation of the project and on the subjects;
- the measures taken to prevent these undesirable events.

a) Notification of premature termination of the project

Whether the premature termination of the study is temporary or permanent, the principal investigator will submit to the ethics committee and MINSANTE a report containing the following information:

- the reasons for the termination of the project (abandonment or interruption);
- the number of subjects recruited to date;
- the measures taken to notify the recruited subjects;
- the repercussions of the termination of the project on the subjects and the measures taken to remedy them;

- summary results.

The researcher must keep research files for a period of at least 10 years [26].

❖ Description of the procedure

- the CERSH and the MINSANTE, each as far as it is concerned, have the responsibility for establishing the monitoring schedule for each research project during its initial evaluation;
- the principal investigator of the research project is responsible for informing the CERSH and the MINSANTE of important discoveries that would affect the risk/benefit ratio [1];
- the principal investigator is responsible for complying with the procedures and deadlines for submitting reports;
- in response to the annual report, CERSH and MINSANTE must indicate any modification, suspension or termination of the initial decision, or confirm that this decision remains valid.

❖ Procedural documents

The principal investigator has the duty to present at least the documents:

below:

•- notifications of modifications to the research project or the documents linked to it;

- an end-of-project report whether premature, temporary or definitive;
- notification of any adverse event occurring during 1 research;
- any other documentation deemed relevant.

Traceability of the latter. The monitoring team also aspires to assume an educational mission with the research teams so that they integrate ethics into the daily reality of the progress of their research project. These aspects are ensured by joint teams, representing MINSANTE and CERSH. A follow-up control decision must be communicated to the applicant.

Monitoring can be passive and/or active. Thus, monitoring mechanisms may include:

■ reports from principal investigators;

■ reports from independent bodies (such as a Data and Safety Monitoring Committee);

■ reviewing reports of adverse and/or unexpected events;

■ meeting the researcher(s) to question them about the recruitment of research subjects;

■ unannounced inspections or supervision of research sites, data, records or documentation of consent, observation of the implementation of the consent procedure [1];

■ interviews with research participants and /or feedback from them.

IV.1.1. Passive continuous monitoring

Initiator: Any principal investigator whose project has been approved

by the, CERSH and MINSANTE

Speaker(s): CERSH. MINSANTE

All principal investigators must submit progress reports to the CERSH secretariat having issued the ethical clearance and to MINSANTE.

CHAPTER IV: MONITORING THE IMPLEMENTATION OF RESEARCH PROTOCOLS AND MONITORING-EVALUATION OF THE FUNCTIONALITY OF COMMITTEES OF ETHICS

Any research project must be implemented within 12 months following approval from CERSH and MINSANTE. If the study does not start within 12 months, the principal investigator informs the ethics committee and MINS ANTE in writing of the delay in starting.

At the discretion of the committee, the favorable ethics opinion may be revoked and the principal investigator will be required to reapply once the issues related to the study delay have been fully resolved; or additional time may also be granted.

IV.1. CONTINUOUS ETHICAL MONITORING DTJN RESEARCH PROJECT ALREADY APPROVED

All approved research projects must be continuously monitored from the date the approval decision was made until the end of the research; follow-up control intervals, to be determined according to the nature of the studies, but at least once a year [13].

The aim of this monitoring is to maintain compliance with the ethical principles of ongoing projects by ensuring that the work corresponds to the research protocol approved by the research regulatory authorities. As a result, it will ensure the protection of participants and researchers as well as the

-In case of minor improvement, an acknowledgment of receipt of the signed amendment by the Head of Division. Operational Research in health is sent to the researcher after notification of the latter,

❖ **Documents of your procedure**

- the letter of notification of the amendment to MINSANTE;

- the letter of approval of the CERSH amendment or the new ethical clearance;

- the new version of the amended protocol;

- acknowledgment of receipt of the C/DROS notification if applicable;

-the new AAR-for major amendments.

-the principal investigator justifying the necessity of this transfer of material provided MINSANTE with a material transfer agreement;

-the DROS verifies the material transfer authorization of the principal investigator;

- the principal investigator provides an annual written update on the use and results obtained from biological samples exported out of Cameroon to DROS.

❖**Procedural documents The Material Transfer Agreement**

III.2.5. Updating an AAR
Initiator: Principal investigator

Speaker(s): MINSANTE (DROS)

For each research project, MINSANTE issues only one Administrative Research Authorization.

❖ **Description of the procedure**

In the event of an amendment, the Principal Investigator must notify the Ministry in charge of Public Health if the amendment has received approval from the CERSH. This notification will be in the form of a letter to which will be attached a copy of approval of the CERSH amendment or the new ethical clearance, and the new version of the amended protocol, dated with a version number including a summary of the modified pages of the initially approved documents, where the changes made to the approved version will be clearly shown.

- In the event of a major amendment, an update following the AAR processing procedure is initiated.

- facilitates the strengthening of local capacities in research and access to care;

- respects cultural values.

Scientific value

The administrative evaluation of a research protocol also focuses on its scientific value. It ensures that the stated objectives are consistent with the methodology proposed in the protocol as well as the expected results.

❖ Guarantee of sharing of research results by the investigator main

The administrative evaluation finally ensures that the principal investigator presents guarantees of sharing the results of the research. This guarantee materializes through;

- the letter of commitment on the honor of the principal investigator to return the research results;

- taking into account/planning the restitution in the timetable for implementing the study activities;

- inclusion of the cost of restitution in the study budget.

III.2.4 Management of biological materials

Initiator:	Principal investigator
Speaker(s):	MINSANTE (DROS)

❖ **Description of the procedure**

In the event that it is necessary to export biological samples outside

from Cameroon (11 •

❖ Checking the authenticity of documents [24]

Authenticity is essential for the following documents:

-Ethical Clearance;

-Agreement of the manager of the site where the study will be carried out;

-the Insurance Policy duly issued by an approved insurance company;

-Marketing Authorization (AMM) issued by DPML/MINSANTE.

The following authentication elements must appear: Title of the protocol/Name of the medicinal product /Reference number/Date of signature/Signing authority/Official stamp.

❖Verification of social and scientific values

The administrative evaluation of research protocols is concerned also to the social value and scientific relevance of the research project.

■ Social value

The social value of a research project is based on the principle that health research must improve the living conditions of humans in the community and in the environment.

The administrative evaluation makes it possible to verify that the research protocol:

-is in compliance with the national health policy and regulations in force;

-justifies a benefit for the community taking into account the risk/benefit ratio;

-protects the research participant;

-the principal investigator withdraws his AAR from DROS against discharge,

Any rejection of a file must be duly justified and notified to the principal investigator against discharge. The re-evaluation of the file is conditional on taking into account the observations which motivated the rejection.

❖Procedural documents

-the Ethical Clearance of the competent CERSH;

-the agreement in principle from the manager of each site where the study will be implemented;

-an import exemption in the case of clinical trials using medicines that do not have a marketing authorization (MAA);

-a valid insurance policy for participants and, where applicable, for researchers during the period of the clinical trial;

-The AAR signed by MINSANTE.

III.2.3. Elements of administrative evaluation of protocols research

The administrative evaluation makes it possible to assess compliance with the principles ethics, protection of participants, institutional compliance, value social and scientific, the commitment of 1 5 principal investigator to restore the results of the study [5].

❖ Verification of compliance with ethical principles

The administrative evaluation also includes control of the principles ethics to ensure the protection of participants. 11 is a double measurement to guarantee the effective protection of participants after the assessment made by the ethics committee.

❖ Checking the authenticity of documents [24]

Authenticity is essential for the following documents:

-Ethical Clearance;

-the agreement of the manager of the site where the study will be implemented;

■the Insurance Policy duly issued by an approved insurance company;

-Marketing Authorization (AMM) issued by DPML/MINSANTE.

The following authentication elements must appear: Title of the protocol / Name of the drug / Reference number / Date of signature / Signing authority / Official stamp.

♦Verification of social and scientific values

The administrative evaluation of research protocols is concerned also to the social value and scientific relevance of the research project.

■ Social value

The social value of a research project is based on the principle that health research must improve the living conditions of humans in the community and in the environment.

The administrative evaluation makes it possible to verify that the research protocol:

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- protects the research participant;

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Any rejection of a file must be duly justified and notified to the principal investigator against discharge. The re-evaluation of the file is conditional on taking into account the observations which motivated the rejection.

♦Procedural documents

-the Ethical Clearance of the competent CERSH;

-the agreement in principle from the manager of each site where the study will be implemented;

-an import exemption in the case of clinical trials using medicines that do not have a marketing authorization (MAA);

-a valid insurance policy for participants and, where applicable, for researchers during the clinical trial period;

-The AAR signed by MINSANTE,

III.2.3. Elements of administrative evaluation of protocols research

The administrative evaluation makes it possible to assess compliance with ethical principles, protection of participants, institutional compliance, social and scientific value, and the commitment of the principal investigator to report the results of the study [5].

❖ Verification of compliance with ethical principles

The administrative evaluation also includes the control of the principles ethics to ensure the protection of participants. This is a double measure to guarantee the effective protection of participants after the evaluation made by the ethics committee.

♦When the research is multicenter or when it provides for the transfer of data or biological research material between several institutions, the following documents are required:

- a data transfer agreement (Data Sharing Agreement);
- a Material Transfer Agreement.

III.2.2. Processing an administrative research authorization request

Initiator: Principal investigator

Speaker(s): MINSANTE

❖Description of the procedure

Immediately after obtaining ethical clearance:

- the principal investigator submits the complete physical application file for Administrative Research Authorization (see 3.2.1 Composition of the AAR application file) to the Ministry in charge of Public Health of Cameroon against discharge;
- the investigator must send by email, the scanned copy of the deposit receipt and said file in one (01) single PDF document to the DROS address:minsantedros@vahoo.com ;
- the file submitted to the MINSANTE mail is sent to the DROS for processing;
- after examining a compliant file, the DROS gives its opinion to the Minister in charge of Public Health within 14 working days of its receipt;
- the approval of the AAR signed by the Minister of Public Health returns to the DROS secretariat who informs the principal investigator;
- the informed consent form of the research participant in French and English;
- Ethical Clearance issued by a CERSH;
- the dated and signed Curriculum Vitae of each investigator taking part in the research;
- the detailed budget, sources of financing and full address of the promoter and sponsor of the project;

- the commitment on the honor of the Principal Investigator to submit to the control of the monitoring-evaluation bodies of the competent authorities, and to report the results at the end of the project
- the plan for disseminating the results of the work; ,
- the prior agreement in principle signed by the head of each health structure/institution where the study will be implemented,

♦For the specific case of clinical trials, the following documents will also be required:

- a photocopy of the insurance policy valid for participants and, where applicable, for researchers during the period of the clinical trial;
- the sponsor's commitment to caring for subjects who are victims of incidents/accidents linked to their participation in the clinical trial and which may occur in the medium or long term;
- the investigator's brochure concerning the product subject to investigation and/or its information leaflet if the 4th product has a marketing authorization.
- letter of referral to CERSH from the principal investigator;
- CERSH evaluation reports;
- the minutes of the CERSH session;
- the document notifying the CERSH decision.

III.2. ADMINISTRATIVE EVALUATION OF RESEARCH PROTOCOLS

Initiator: Principal investigator

Speaker(s): MINSANTE

The administrative evaluation of research projects falls under the jurisdiction of the Division of Operational Research in Health (DROS) of the Ministry of Public Health [5, 24, 25],

III.2.1. Composition of the Administrative Research Authorization application file

The documents listed below must be submitted in order in one bound document:

- the request for Administrative Research Authorization (AAR) addressed to the Minister in charge of Public Health signed by the Principal Investigator or the head of the research institution;
- the research protocol dated and signed by the Principal Investigator, approved by the sponsor or promoter, and including the address of the

natural or legal person submitting the request (postal box, fax, telephone, email address and the headquarters of the institution or research center);
- the research participant information sheet in French and English;
- the permanent/technical secretary notifies the chairman of the resubmission committee;
- the chair of the committee decides whether the protocol requires an expedited review or a normal evaluation by the committee;
- if the amendment is major, the resubmission is scheduled for the next CERSH session to be evaluated. If the amendment is considered minor, it is assigned to an evaluating member who presents the report during the session;
- the members of the CERSH decide on the proposed major amendment. The decision taken on this subject by the members will be transmitted to the principal investigator;
- the technical secretariat will make the minutes of the session available;
- the Permanent/Technical Secretary maintains all documents and correspondence submitted for amendments.

b) Appeal after an unfavorable opinion from the CERSH

Any principal investigator may appeal an unfavorable decision or of a request for compulsory modification which he considers not justified.

❖ **Description of the procedure**

- The principal investigator must send a request to the chairman of the committee in which he justifies his disagreement within a maximum period of thirty (30) days upon receipt of the CERSH's decision;
- the chairman of the committee must respond to the principal investigator within four-five (45) days from the date of referral. This answer could be an explanation, written upon the complete or partial re-evaluation of the application file. The latter is required to respond either in writing or during a hearing.

Minor amendments are administrative corrections in the protocol, changes in promoter address, etc. They must be notified to the various ethics committees concerned.

a. Resubmission of an already approved application/Amendments

Changes cannot be made to the research project without prior approval of the ethics committee.

❖ **Description of the procedure**

- The principal investigator submits to the CERSH a request for approval of modification containing the following elements:
- a letter addressed to the chairman of the committee explaining the reasons for the amendment;
- a list of amendments and their locations in the protocol/informed consent form;
- the modified pages of the initially approved documents, where the changes made to the approved version will be clearly described (track changes function);
- the new version of the amended protocol, dated with a version number
- description of ethical considerations related to the modification;
- the declaration of all steps taken with other ethics committees and their decisions with a view to obtaining approval of the modified documents;
- administrative research authorization if applicable;
- the report of the protocol steps already carried out if applicable.

❖Documents supporting the procedure

- the document bearing the final decision of the CERSH (ethical clearance, information letter, exemption letter);
- the mail discharge register of the CERSH secretariat.

III. 1.10. Resubmission of a research protocol

Initiator: Any principal investigator requesting a resubmission of the research project to CERSH

Speaker(s): CERSH

A protocol can be resubmitted in the event of [13]:

- major changes that impact the risk/benefit ratio or the information contained in the consent form;
- unfavorable opinion of the ethics committee;
- amendments.

An amendment is considered major when it has a significant impact on the objectives, hypotheses and/or study design, the site of implementation of the study, changes in the research team, the safety of the participants, the interpretation of scientific data and the quality of the medical product investigated in the study/ clinical trial.

Other changes requiring resubmission for amendment are: changes to the protocol after approval, changes to the research team, addition of new research sites or extension of the project period, changes to the targets of the research, and ethics retained by the committee to the principal investigator at the latest, within the fifteen (15) days after deliberation of the submitted research protocol.

The table below summarizes the transmission times and the elements of the decision press release specific to certain contexts.

Table 5: specific contexts, types of decisions, content of the document and deadlines for transmitting the document containing the CERSH decision

Contexts	**Type of Decision**	**Deadline for the transmission of the decision**	**Content of the document communicating the decision of the CERSH**
	Favorable opinion without modifications to be made to the submitted research project. **Favorable opinion** subject to modifications recommended by the CERSH. Unfavorable opinion for the implementation of the submitted research project.		i) exact title of the research protocol examined: ii) ii. Clear identification of the research protocol or proposed amendment: names, dates and version number of reviewed: documents iii) iii. Suggestions possibly made by the committee by information letter structured according to the ethical review plan; iv) Final decision; v) Clear justification for disapproval and exemption from ethical clearance.
Case of emergency	Waiver/Exemption from ethical clearance.	72 hours	
Ordinary session	Same as in emergency situations.	15 hours	Same

- during the evaluation process, the CERSH may request additional information
- information or modifications to be made to the protocol before issuing an opinion;

- in the event of reservations expressed on the merits of a protocol, another examination by the CERSH is required,

At the end of the evaluation of the validity of the scientific and ethical aspects of the research protocol by the CERSH, the following results are possible:

- favorable opinion without modifications to be made to the submitted research project;
- favorable opinion subject to modifications recommended by the CERSH;
- unfavorable opinion for the implementation of the submitted research project;
- ethical clearance exemption/dispensation".

Any decision of the ethics committee must be justified and notified in writing to the tenderer.

In the event of an unfavorable opinion, the principal investigator may appeal in writing to the chairman of the committee (3.1.10.2. Appeal after an unfavorable opinion from the CERSH).

❖Documents supporting the procedure

- minutes of the research protocol evaluation session;
- ethical clearance or information letter on the CERSH decision.

III. 1.9. Communication of the CERSH decision

Initiator: Permanent or technical secretary of CERSH

Speaker(s): CERSH technical secretariat, investigator main

❖ Description of the procedure

- the principal investigator must justify that his research project meets the above criteria;
- in session, the CERSH evaluates the request and decides whether or not to grant an exemption from evaluation;
- the principal investigator is notified whether or not the ethical clearance exemption has been granted by the CERSH;
- the principal investigator who has obtained an unfavorable response may submit an amendment, or request their participation in the next session of the committee to provide further explanations.

❖ **Documents supporting the procedure**

- evaluation grid for the research protocol;
- notification of exemption from ethical clearance signed by the President,

III.1.8. CERSH decision making

Initiator: , Chairman of the committee

Speaker(s): CERSH members

- All members of the CERSH must have the opportunity to express themselves and be invited to do so [13]. They are bound by confidentiality regarding the deliberations held. A commitment must be signed to this effect during the selection of members.vau CERSH,

❖ **Description of the process**

- after presentation of the evaluation report by the members of the CERSH and/or independent experts who have them. given the responsibility, a consensus is desirable for decision-making. Failing this, a vote is organized and the decisions are taken by simple majority;
- if the study involves secondary data from patients, these data may be used provided that they are anonymous and for confidential use,

❖**Documents supporting the procedure**

- rationale for waiving informed consent in the protocol;
- evaluation grid for the submitted protocol;
- minutes of the session;
- summary reports evaluating the protocol submitted.

III. 1.7. Ethical clearance exemption

Initiator: Any principal investigator whose research project does not involve explicitly obtaining ethical clearance

Speaker (s): CERSH

Certain studies may be exempt from ethical clearance in particular;

- studies using published and authorized online databases as part of research;
- studies using coded data (anonymous databases, coding of individual names);

- studies relating to the review of documents published or placed in the public domain (scientific articles, laws and regulatory texts, directives, etc.);
- studies relating to the evaluation of a health program/service or a care protocol whose results will only be used to improve the performance of the program/service concerned.

NB: The decision to waive ethical clearance is nevertheless taken by the CERSH after analysis.

- if obtaining consent could compromise the result of the study (use of anonymous data, use of anonymous biological material, ecological and migratory type studies where the target is a group and not an individual);
- whether the study involves secondary data;
- if individual data is not collected.

In the event that the study meets the above situations, the researcher must request from the CERSH, an exemption from informed consent.

♦Description of the procedure

- the principal investigator must justify that his research project complies with the criteria raised;
- in session, the CERSH evaluates the request and decides whether or not to grant a waiver of consent;

To this end, the evaluators must verify that:

- the use of registers/databases is authorized by the institutions which host them;
- the confidentiality procedure used makes it possible to protect access or use of personal information in the registers/database;
- the exemption from informed consent does not affect the rights and well-being of research participants ;
- the study concerns biological samples and/or personal data collected as part of research with prior informed consent from the participants for/subsequent use of their data/samples;
- technology, medical benefits or it 1 participation in intellectual property rights);
- a data security and monitoring plan is attached to the protocol;
- the process of obtaining and documenting consent follows standards;
- the principal investigator must provide the investigator's brochure: preclinical data (animal experimentation) and results of early phase clinical trials (if applicable).

III. 1.5. Evaluating the researcher's conflict of interest situation
Regardless of the type of research, reviewers will need to ensure that the researcher has declared a conflict of interest [7]. See the procedure in section [2.2.14], Managing conflicts of interest during ethical evaluations of research protocols.

III. 1.6. Exemption from informed consent
Initiator: Any principal investigator whose study does not involve obtaining informed consent for the use of information, data and/or samples

Speaker (s): Members of the CERSH

Situations for exemption of informed consent for study

The situations described below may lead to an exemption from informed consent for research.

- If it is not possible to access the participant.
- If the risk of violation of autonomy and confidentiality is minor
- The tests to be conducted on the samples are listed and limited to addressing the research questions outlined in the protocol.
- there is a clear justification for the need to transfer the samples (verify that there is no possibility of doing the necessary tests locally);
- a material transfer agreement (Material Transfer Agreement) is signed between the sample collection site, the research team that sends the samples and the team that receives the samples;
- The material transfer agreement is drawn up according to the attached outline,

d) Specific criteria for evaluating research protocols on Improved Traditional Medicines (MTA)

For the particular case of clinical trials with phytotherapy products or. any other alternative therapy, the evaluation of research protocols is only done for MTA categories 3 and 4. This categorization is made by the DPML.

The CERSH, in addition to the elements required for an ethical clearance request, examines and assesses the following elements [23]:

- the composition of the research team includes at least one person with skills in clinical medicine;
- there exist plans for preservation of traditions practices and intellectual property rights;

- compliance with international standards on access and benefit sharing (APA):
- the existence of a plan for sharing profits in the event of commercialization of the product or research process or obtaining a patent (transfer of
- the research site: the capacities of the site in terms of technical platform to respond to emergencies that may arise from the implementation and monitoring of the intervention within the framework of the study;
- notification, investigation and response to adverse events: circuit compatible with the national adverse event notification system; investigation procedure to establish the attributability of serious adverse events (SAEs) and signals; protocol for handling these;
- certification of an insurance policy that covers participants throughout the period of the study and the availability of resources for its renewal and validity;
- the functioning of the Data Safety Monitoring Board (DSMB): its composition (mandatory presence of experts who can understand and investigate SAEs), the independence of its members in relation to the investigation team, an appropriate frequency of meetings for monitoring SAEs, members' access to the blinding procedure; the conditions and decision-making power to stop the clinical trial in the event of a major risk detected;
- monitoring and auditing of the trial: checking whether there is an internal and external monitoring plan; a plan for using the results of this monitoring to improve the results of the trial; an audit planned at the end of the test.

c)Specific criteria for evaluating research protocols involving transfer of biological materials between teams of investigators [7]

The committee must ensure during the evaluation that:

- the transfer makes it possible to answer a research question;
- the attributability of serious adverse events and the signals and protocols for their management;
- certification of an insurance policy that protects participants throughout the period of the study and that resources are available for its renewal and validity;
- the adequacy of the composition and functioning of the Data Safety Monitoring Board (DSMB) (For phases 1, 2 and 3): the mandatory presence of experts who can understand and investigate serious adverse events , the independence of members from the investigation team, an appropriate frequency of meetings for monitoring serious adverse events, members' access to the blinding procedure ; the conditions and

decision-making power to stop the clinical trial in the event of a major risk detected;
- the existence of a monitoring plan and internal and external audit of the trial: a plan for using the results of this monitoring to improve the results of the trial and an audit planned at the end of the trial essay.

d) Specific evaluation criteria for clinical trials involving medical interventions [22]

The assessor must check:

- the description of the intervention tested: detailed procedure in terms of actions and equipment, the results of previous experiences, the inventory of risks that may arise and response measures, the advantages compared to current standards;
- the description of the control intervention: detailed procedure in terms of actions and equipment, the inventory of risks that may arise and response measures, the advantages compared to current standards;

NB: The agreement document must be signed by the following persons: the sender, the recipient and the sample security officer.

❖Criteria for evaluating research protocols for special cases a) Criteria for evaluating research protocols for clinical trials with intervention product [22]

For this particular case, it will be a question of comparing the foreseeable disadvantages with the expected advantages for the participants and the communities concerned in the research. To this end, the evaluators will verify that:

- the investigator's brochure contains all the information (as presented in Table 3);
- the review of the medication is complete and systematized: description of the galenic medication, doses and storage conditions; person in charge of administration, adequacy of stock in relation to demand, quality control system, supply chain and stock management, procedures for destroying the drug at the end of the trial;
- the description of the control drug and/or placebo corresponds to the standard protocol/drug, justification for the use of a placebo;
- the research site has the capacity in terms of technical platform to respond to emergencies that may arise from the first administration of the product to participants (phase 1) and for the implementation and monitoring of the intervention as part of the study (phases 1,2,3 and 4);
- the notification, investigation and response to adverse events are clearly described: circuit compatible with the national adverse event notification system ;

- investigation procedure to establish
- The data sharing agreement must provide that:
- the collected database is hosted in Cameroon;
- procedures for securing the collected data exist (archiving method);
- the order of authors in publications is precise and fair;
- access to the collected database is equitable (access of local investigators to the database of all sites of a multi-country or multicenter study).

■The material transfer agreement

It must be signed for studies involving an exchange of biological material between the intervening institutions. The evaluators will ensure that this agreement includes at least the following information:

- the nature, quantity and frequency of sample transfer;
- the description of the samples in terms of source, volume, method and duration of storage and transport;
- the name and address of the sender and his institution;
- the name and address of the recipient and their institution;
- the method of destruction;
- the conditions for processing derived products;
- the conditions for sharing with other laboratories;

equity in the sharing of profits generated by products derived from the biological material transferred (intellectual property, operating patent and marketing);

- the law applicable to sample security and the referring person. available and relevant information on risks and benefits to maintain their informed consent;
- The procedure includes provisions for receiving and responding to concerns/complaints from participants or their representatives before and during the study.

f) Respect for confidentiality

The evaluators must ensure that the data collection tools and the procedures for collecting, analyzing and presenting the results do not allow the identification of participants.

To do this, they will check that:

- access to the databases is restricted to the research team;

- the coding system is appropriate (in case there is a code register).

g) Collaborative partnership to improve access to research results

The guarantee of sharing of research results must be ensured [1, 7].

The evaluators will have to verify that the research protocol provides for a collaboration plan between the actors (government, sponsor, health authorities and community), guaranteeing the populations' access to the results of the research on the financial and technological levels (technical platform) and training of human resources.

If the data comes from and/or is used by several institutions, evaluators will verify that there is a data sharing agreement signed by all stakeholders as well as a plan for disseminating the research results.

❖ Consent among vulnerable groups/individuals

A vulnerable person [1, 7] is any person presenting a situation

health, physical, social, and/or economic which does not allow them to assume their autonomy and/or to freely give consent (children, pregnant women, newborns, fetuses, young homeless people, people with mental disorders /psychiatric, economically or educationally disadvantaged, marginalized, people in terminal illness, prisoners etc.).

Apart from the information notice containing general information, the assessors will check that:

- the study provides a consent form allowing the legal representative to give permission for the person to participate in the study;

- the study gives the vulnerable participant who has regained their autonomy the opportunity to give their consent.

♦ Consent of participants in medical emergency [1]

Here, we are talking about predictable emergencies (people with emergencies not foreseeable are classified among the vulnerable groups).

Assessors must verify that the collection of consent is planned before the emergency occurs.

e3) Monitoring of informed consent

The procedure for obtaining informed consent must also describe the manner in which monitoring will be carried out during the period of implementation of the research project.

To do this, the evaluators will check that:

- the procedure provides for arrangements for participants to receive during the study (clinical trials and cohorts) and at a defined periodicity

- the informed consent form contains the following information: title of the project, the sentence of acceptance, the space for the name of the participant, the space for the signature of the participant and any witnesses;

- The language used on the consent documents is simple and understandable to the participant.

❖ Consent for children under 12 years old

The evaluators will verify that:

-the information notice containing general information intended to obtain consent is offered to the parent or guardian;

-the parental consent form is offered to the child's parent/guardian.

❖ Consent for minors aged 12 to 20

The evaluators will verify that:

-the information notice containing general information intended to obtain consent is offered to the parent or guardian;

- an assent form is offered to the child; this form must include the following information: title of the project, acceptance sentence, space for the name of the participant, space for the signature of the participant and any witnesses;

-a parental consent form is available.

❖Consent of an emancipated minor

According to the provisions of the civil code applicable to Cameroon [21], is considered as an emancipated minor any individual under the age of 21 (minor) who is married, or declared as such by a legally valid decision of the parents from the age of fifteen.

In this case, the evaluators will ensure that the study plans to apply the same procedure for obtaining consent as in adults.

- the place where consent is collected respects confidentiality, comfort and participant safety;
- if the study provides for self-collection of consent, that a member of the team read and explain to the participant;
- the language and language used is appropriate and understandable to the participant;

The consent collection procedure allows the participant to ask questions for clarification before participating.

In addition to the general conditions cited above, the following specific criteria will be the subject of the evaluation of the process of obtaining informed consent from participants in a study,

e2) Specific conditions

❖ Consent from adults

The evaluators will check that:

- the information notice includes the following information: title of the project, name and full address of the principal investigator, objectives, study procedures (mode of administration of the questionnaire, sample collection, sampling, randomization, data analysis, duration of data collection and monitoring, frequency of visits), risks and benefits of the study, information on the voluntary nature and refusal without consequences of the subject's participation in the study, possible compensation and the justification of the amounts, the possibility of withdrawing consent the address of the people to contact in case of concerns; the study provides that a copy of the notice be given to the participant;

e) The process of obtaining informed consent

It varies depending on the type of participant and the data source. We thus distinguish:

- direct data sources: adult (21 years and over); child (under 12 years old); adolescent (12 to 20 years old); vulnerable people; participants in emergency situations;
- indirect data sources: patient files, database, serum library or biobank;
- a community or group.

e1) General conditions

Regardless of the type of study participant and the data source, the evaluation of the process of obtaining consent from research participants will focus on verifying the following aspects:

- the relevant consent/assent documentation is available (information notice, informed consent form, assent form, parental consent form), written in French and English;
- the procedure for obtaining consent is clearly described in the protocol and provides for translation into local dialects where appropriate;
- this procedure does not expose the participant to : subordination, induction, pressure and/or exploitation ;

- the procedure offers the participant the possibility of accepting or refusing their consent for the use of their data or samples for further studies;

To this end, evaluators must:

- make an inventory of the expected benefits (improvements in efficiency, safety or improvement in accessibility to care), which will cover the following aspects: individual (physical, physiological, psychological, emotional, dignity and rights); social (disruptions to family, social and cultural norms and organizations) and environmental that may have an impact on individuals or society;
- judge whether the benefits are reasonable in relation to the risks;
- identify the direct and indirect benefits of the study;
- check if there is any alternative or procedure that could improve the risk ratio /benefit.

d) Fair selection of participants

The choice of research participants must be done fairly. To do this, evaluators must check whether:

- the eligibility criteria are clearly defined;
- the criteria for withdrawal or interruption of participation in the study are clearly defined;
- all strata that can benefit from the research will be eligible for recruitment;
- special protective precautions for vulnerable populations are taken;
- the eligible or included people correspond to the direct beneficiaries of the research;
- people with the greatest risk of harm are excluded;
- the selection of participants is made so as not to influence the result.
- the product has been the subject of a clear description, the phase of the test is specified;
- the results of previous phases are clearly described;
- the choice of variables of interest is justified.

If the research requires paraclinical examinations, ensure that a quality control procedure is planned and described.

If applicable to make sure that:

the instrument to be tested is annexed or the elements to be collected are at least described;

- the pre-testing of the instruments is described.

The evaluation of scientific validity also makes it possible to specify in which perspective the research is located and what type of study it is:

- observational research (descriptive, analytical, etc.);
- interventional research (trial: therapeutic trial, vaccine trial, evaluation of biomedical equipment, evaluation of a medical device or medical technique);
- research/ development;
- national, regional or institutional study;
- multi-country study.

c) Risk-benefit analysis

It is a value judgment between the risks incurred by the participants, the morbidity and mortality and the socio-economic impact of the targeted health problem and the expected contribution of the protocol to be evaluated.

- random verification of informed consent forms kept by the researcher in participant files;
- on-site examination of the measures taken to ensure the confidentiality of information;
- visiting research sites;
- the periodic review of documents produced as part of the research project (questionnaires, consent forms, serious event reports).

❖ **Description of the procedure**

- the supervisor and the principal investigator agree on the date, time and location of monitoring, as well as the necessary documents;
- using the checklists/monitoring grid, the supervisor examines part or all of the aspects of the methods of participation in research (see appendix no. 2);
- after a follow-up visit, a meeting is held with the principal investigator to inform them of any findings;
- the collected data is then analyzed and a report is written, all comments, recommendations and appropriate corrective and preventive measures (if the monitoring reveals problems or deficiencies) are included in the report which will be sent to the principal investigator
- the written report is simultaneously kept at CERSH and MINSANTE.

The report of any new information on elements likely to increase the risks and inconveniences for the subjects, to harm the smooth running of the project or to have an impact on the desire of a subject to continue to participate in the project must contain the following information :

- the name of the authority carrying out monitoring or control;
- he nature of the new information/issues noted;
- the number of subjects affected by these elements;
- the repercussions of these elements on the continuation of the project and on the subjects;
- corrective and preventive measures;
- any external promoter must also be informed of the findings;
- The investigator has the opportunity to respond in writing to the findings of the report, offering additional supporting evidence and/or taking required actions specified in the report.
- If corrective and preventive measures are not taken into account or if an immediate risk is detected, a temporary cessation of the study may be considered.

❖ **Procedural documents**

- memo organizing supervision
- proof of notification/information of follow-up to the principal investigator;
- completed monitoring grids;
- attendance records/lists of persons supervised;
- supervision/monitoring reports;
- proof of notification and transmission of the investigation report to the principal investigator, the CERSH secretariat, MINSANTE and the promoter/sponsor of the research.

IV.2. TRACEABILITY/STORAGE OF RESEARCH PROTOCOL IMPLEMENTATION DATA

Initiator: CERSH Secretariat, MINSANTE

Speaker (s): CERSH, DROS

For CERSH and MINSANTE, this involves ensuring the traceability of each research work carried out [1].

❖Description of the procedure

- All documentation relating to the implementation of a research project must be recorded in a database in digital media (USB key), at the secretariat of the ethics committee and at MINSANTE.
- This database will contain at least the following information:

- contact details of the principal investigator and co-investigator(s);
- the composition of the research team;
- the references of ethical clearance and administrative research authorization;
- research sites;
- the key stages of the final timetable for implementing the research work;
- proof of restitution of research results (scientific articles, abstracts of scientific conferences, report of restitution sessions, etc.);
- ethical and scientific evaluation report of the project;
- any document submitted by the researcher during the project, including documents relating to ongoing monitoring;
- any exchange between the ethics committee and the research team or the parties concerned;
- any exchange between the members of the ethics committee relating to the research project.
- A complete physical file including all documentation related to a research protocol must also be kept by the CERSH secretariat and MINSANTE.

❖**Procedural documents**

- digital support ;
- complete physical file of the research protocol.

IV.3. ACTION TO BE TAKEN IN CASE OF INTEGRITY PROBLEMS AND NON-COMPLIANCE WITH ETHICS IDENTIFIED BY CONTINUOUS MONITORING.

When a supervisor/reviewer finds reason to believe that continuing a research project will compromise the welfare of participants [1], regulatory authorities and/or the ethics committee may:

- respectively withdraw the AAR and/or ethical clearance from a research project if necessary;
- report to administrative or state authorities any serious or repeated violation of ethical standards;
- request from the competent authorities the suspension of authorization to receive funding for research, to perform experimental interventions, or authorization to practice medicine;
- request scientific journals which have published the results of a study, to withdraw any article which subsequently turns out to be based on falsified data or which is based on unethical research.

IV,4. MANAGEMENT OF REQUESTS, NON-COMPLIANCE OR DEVIATION OF PROTOCOL

Initiator: CERSH, DROS Secretariat

Speaker (s): CERSH, MINSANTE

Following the monitoring and evaluation activities of the implementation of a research project, the CERSH or the MINSANTE may require the modification of the research, suspend or withdraw the ethical and administrative approval of the research [1].

- Suspension of an Ethics Clearance/AAR means that all project activities must cease (subject recruitment, data analyses, and publication of existing data) until all outstanding issues are resolved. satisfactory manner. Suspended studies are still approved, but in a " pending " status until " that outstanding issues are resolved.
- Withdrawal of an Ethical Clearance/AAR means that the study is no longer approved. All project activities must cease immediately, including data analysis. Any resulting data or analysis is void.

The following situations may be sources of concern for MINSANTE and CERSH:

- the continuation of the research does not make it possible to respect an acceptable balance between the risks and the potential benefits for the participants;
- the research is not carried out in accordance with the protocol approved by the authorities and regulatory requirements;
- the research is associated with serious and unforeseen harm to participants (which may have been determined following a review of reportable events by CERSH or MINSANTE (DROS/DPML) or monitoring committee reports on data security/data safety monitoring board);
- falsification of research files or data;
- non-compliance with conditions previously imposed by CERSH or MINSANTE;
- repeated or deliberate failure to obtain consent or properly document the consent process from research participants;
- repeated or deliberate non-compliance with the conditions imposed by the CERSH, the promoter or MINSANTE;
- repeated or deliberate non-compliance with obtaining approval by CERSH and MINSANTE, modifications or changes made to the research protocol;
- repeated or deliberate non-compliance with the proper maintenance of research records or the reporting of events required to be reported to the CERSH/regulatory authority.

- The President of the CERSH is not authorized to withdraw approval of research without consultation with the other members of the ethics committee.
- However, it is authorized to suspend research approval in cases of emergency when the rights, safety or well-being of subjects are in immediate danger. This suspension must then be declared respectively to the CERSH during its next plenary session and to a commission of experts constituted by MINSANTE. The CERSH and/or the MINSANTE may withdraw their approval after studying the question in plenary session with the members of the committee or commission.
- The decision to suspend or withdraw research approval by CERSH/MINSANTE must take into account the safety, rights and well-being of participants already registered in the research. It also takes into account the measures to be taken to guarantee the quality of care received by the participants, the measures taken to notify the participants of the suspension or termination of the research, as well as the appropriate time to transmit this information to them.

❖ **Description of the procedure**

- the president of the CERSH or the Minister of Public Health notifies the investigator by official letter of any suspension or withdrawal of approval of the research by the CERSH/MINSANTE, providing reasons for the adoption of this measure and the corrective measures proposed, if applicable.
- the president of the CERSH or the Minister of Public Health informs the establishments/sites hosting the project and the regulatory bodies (if applicable).
- The principal investigator is responsible for timely notifying all co-investigators, the research team and partners associated with the project, as well as the subjects enrolled in the study.
- The principal investigator will be required to develop withdrawal procedures that protect the rights, safety, and well-being of participants and describe them to participants.

❖ **Procedural documents**

- Letter of suspension or withdrawal of Ethical Clearance or Administrative Research Authorization.

IV.6. LIFTING OF THE SUSPENSION OF ADMINISTRATIVE RESEARCH AUTHORIZATION AND ETHICAL CLEARANCE

Initiator: Principal investigator

Speaker (s): CERSH, President of CERSH, Minister of Health, DROS, Principal Investigator

- The suspension may be lifted after corrective measures have been taken to the satisfaction of CERSH, MINSANTE and other bodies involved, if applicable.
- To reinstate a research project that has been suspended, the investigator must "satisfactorily resolve all outstanding issues. After one year of suspension or the study expiration date, if no progress has not been carried out on the outstanding questions, the CERSH and the MINSANTE are proceeding with the withdrawal of the Ethical Clearance and the AAR.

❖ **Description of the procedure**

- The principal investigator must contact CERSH/MINSANTE in writing within 60 days following the suspension. He must submit an application including and meeting the following requirements:
- the reason for the request to reinstate the study;
- a brief summary of the objective of the study and the objectives/expected results; these can be incorporated into the protocol, noting any changes, revisions or clarifications to the protocol;
- description of changes made, if any, since initial approval;
- a summary of the progress of the study including: the number of subjects already enrolled, the implementation phase of the procedures at the time of suspension, any adverse events that have occurred and how these adverse events will be mitigated upon completion future, any relevant additional information;
- a procedure and a sworn undertaking demonstrating that the reason for suspension will not recur and that the study will comply with all applicable laws and regulations.
- the request to lift the suspension is subject to examination during a session of the CERSH and a meeting of the MINSANTE expert commission;
- the examiners analyze and decide according to the known procedure;
- in the event that approval of a protocol by CERSH/MIN SANTE is reinstated, it may be required that subjects who were previously enrolled be re-enrolled;
- notification of the decision is made to the principal investigator; and to any other person concerned (sponsor, complainants, research site managers, others.

♦ Documents of the procedure

- file requesting lifting of the ethical clearance/AAR suspension;
- minutes of the CERSH evaluation and the commission meeting;
- the CERSH report;
- the notification of decision from CERSH and MINSANTE.

IV.7. MONITORING COLLABORATION WITHIN THE TEAM OF RESEARCHERS AND WITH OTHER STAKEHOLDERS IN THE FIELD/RESEARCH SITE

- The responsibility for establishing a research project governance committee lies with the study sponsor. This involves capturing all information concerning the effectiveness of:
- announced research sites;
- collaboration between research teams and research site managers (internal monitoring of the research site) [1,7];
- supervision of research work by the research team announced in the protocol;
- the exchange of data/materials within the research team.

Most collaborations will usually require some of the following to be addressed:

- commitments in terms of financing and in-kind resources;
- the essential tasks and responsibilities of partners;
- project management arrangements, including hierarchical and communication channels;
- training requirements and responsibilities;
- health and safety arrangements for staff;
- authorship of publications;
- Intellectual property ;
- a brief summary of the objective of the study and the objectives/expected results; these can be incorporated into the protocol, noting any changes, revisions or clarifications to the protocol;
- description of changes made, if any, since initial approval;
- a summary of the progress of the study including: the number of subjects already enrolled, the implementation phase of the procedures at the time of suspension, any adverse events that have occurred and how these adverse events will be mitigated upon completion future, any relevant additional information;
- a procedure and a sworn undertaking demonstrating that the reason for suspension will not recur and that the study will comply with all applicable laws and regulations.
- the request to lift the suspension is subject to examination during a session of the CERSH and a meeting of the MINSANTE expert commission;
- the examiners analyze and decide according to the known procedure

- in the event that approval of a protocol by CERSH/MINSANTE is reinstated, it may be required that subjects who were previously enrolled be re-enrolled;
- notification of the decision is made to the principal investigator; and to any other person concerned (sponsor, complainants, research site managers, others).
- the storage, transfer and conditions for future use of data and research samples (including return or destruction);
- specific information governance and security requirements, including record keeping, management, handling;
- financial contributions/resources and responsibilities/compensation.

It will be a question of verifying the existence and knowledge by all stakeholders of the standard operating procedures (SOPs), of the management of the research project.

IV.8. MONITORING AND EVALUATION OF STRUCTURES INVOLVED IN THE IMPLEMENTATION OF RESEARCH PROJECTS

This monitoring is carried out annually by the Division of Operational Research in Health (DROS) of the Ministry of Public Health. This involves ensuring that:

- these structures have the resources enabling them to ensure the protection of research participants during the planning and implementation of human health research projects;
- the implementation of the protocols is done according to national regulations;
- measures are taken to make research results accessible to research beneficiary communities, decision-makers and scientists.

The indicators selected are presented in the supervision grid for sites conducting or hosting human health research projects (see appendix).

IV.9. MONITORING AND EVALUATION OF CLINICAL TRIAL PROTOCOLS BY THE NATIONAL PHARMACEUTICAL REGULATORY AUTHORITY (ANRPH)

The monitoring and evaluation of clinical trial protocols by the ANRPH [27] is carried out jointly between the institutions involved (the General Inspectorate of Pharmaceutical Services and Laboratories, the Directorate of Pharmacy, Medicines and Laboratories, the National Laboratory for Quality Control of Medicines and Expertise, the Directorate of Family Health and the Division of Operational Research in Health), through the committee coordinating its activities.

The table below presents the different performance indicators for monitoring and evaluating research projects.

Table 6: Indicators for monitoring/evaluation of clinical trial protocols from the National Pharmaceutical Regulatory Authority (ANRPH)

No.	Indicators
1)	Proportion of clinical trial protocols implemented in MINSANTE structures with ethical clearance
2)	Proportion of clinical trials implemented in the structures of the MTNSANTE with ethical clearance and AAR
3)	Proportion of AAR requests with response to the requester
4)	Proportion of AAR requests with a favorable response to the applicant
5)	Average length of time between submission of the complete AAR application file and the final response to the applicant
6)	Proportion of special exemption requests for imports of clinical trial drugs processed
7)	Average processing time for exemption request files for the importation of clinical trial drugs

*

8) Proportion of clinical trials whose drugs were imported after special import exemption and which benefited from the lifting of the batch

9) Proportion of clinical trials that have benefited from an administrative monitoring visit in the last 12 months

10) Proportion of phase I or II clinical trials that have benefited from an annual administrative monitoring visit during the last 12 months

11) Proportion of ethics monitoring visits carried out

12) Proportion of regulatory texts and Directives relating to clinical trials revisited over the last 36 months

IV.10. PROCEDURE FOR REVISION/UPDATING OF THIS DOCUMENT

Initiator: MINSANTE, CNERSH,

Speaker(s): All research stakeholders in Cameroon

Responsibility for maintaining and updating this document (Organization, operation and procedures of the ethical and administrative evaluation system for human health research in Cameroon) rests with MINSANTE and CNERSH. The decision to modify existing procedures is taken on the initiative of the DROS and brought to the attention of the Minister in charge of Health. This update must be the subject of a memo from the Minister of Health. Appropriate numbering of the versions relating to the updating of said Document must make it possible to monitor its completeness.

Updating the Document may be motivated by:

- the evolution of the legal framework for research for human health
- the evolution of national and international best practices;
- changes in the organization or activities of technical structures regulating research for human health following the decisions of those responsible;
- the need to modify systems and procedures to deal with new situations;

- user feedback on difficulties in implementing existing procedures.

❖Description of the procedure

When the decision to modify existing procedures or insert new procedures is considered:

- the head of the operational health research division submits to the Minister of Health a project to create a working group made up of stakeholders in the regulation of health research;
- the working group is invited by the Minister of Health to the working sessions;
- the objectives of the review are discussed in the group and are approved;
- subject matter experts write the identified procedures;
- the written or revised procedures are presented to the group for discussion;
- after discussion, the procedures are adopted as well as the new version of the document;
- the new version as well as the meeting reports are submitted to the Minister of Health for validation by a decision.
- the version validated by the Minister of Health is produced in French and English and shared by DROS.
- decision to create a working group relating to the revision of the document;
- working group activity report;
- new updated version of the document;
- decision making the document enforceable.

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APPENDICES

Appendix 1: Ethical evaluation grid for a research project.

Title of Research project

……
……
……
……

Institution:
……
……

Name of Principal Investigation
:…….

Name of Sponsor:
……
……

Type of Project: National International Student

Quality of the Document:
……

Date of Evaluation: ……/……../…….

Type of Evaluation: Normal Accelerated Project amendment Monitoring

EVALUATION CRITERIA				
1. SOCIAL VALUE's	Yes	No	NA	Comments/Suggestions
The theme relevant to population health research objectives?				
Is the project in line with priority national and/or global health needs?				
Does the project have a direct and indirect benefit for the populations?				

Does the plan for disseminating the results involve participants, the community, local and national health authorities, partners working in the relevant health field and the scientific community?				
2. SCIENTIFIC VALIDITY				
COHERENCE OBJECTIVESASSUMPTIONS - QUESTIONSRESEARCH-ANALYSIS PLAN				
Does the research question arise from its justification?				
Do the hypotheses allow the research questions to be answered?				
Do the objectives allow the research questions to be answered?				
Are the proposed study design and the participant selection method suitable to achieve the set objectives?				
Does the study process follow from the objectives and hypotheses? Does the process reflect the characteristics of the subjects being studied?				
Does the analysis plan correctly estimate the indicators of the specific objectives set?				
Do the collection tools meet the objectives? Does each objective have the variables expected in the collection tool?				
2.2. SELECTION OF PARTICIPANTS				

Is the method of selecting participants suitable to achieve the objectives set?				
The selection of participants, the estimation of the sample size and the proposed measurement methods help to minimize selection bias. information, sampling fluctuations and confounding factors?				
Does the study procedure provide for fair selection of participants?				
Do the eligible or included people correspond to the direct beneficiaries of the research?				
Are people at greatest risk of harm excluded?				
Is the selection of participants done so as not to influence the result?				
2.3. SPECIAL CASE OF A CLINICAL TRIAL				
What is the phase of the trial? Specify PHASE 1;2;3 or 4				
Are the results of previous phases described?				
Does the proposed randomization method allow for a balanced distribution of study participants?				
Are the intervention, control and measurements of the variables of interest clear and precise?				
Was the product clearly described? Is the choice of variables of interest justified?				
If the research requires paraclinical examinations, is a				

quality control procedure planned and described?				
2.5 RELIABITLITY AND VALIDITY THE INSTRUMENT				
If applicable: is the instrument to be tested annexed or are the elements to be collected at least described?				
Is the pre-testing of instruments described? Is the instrument available?				
is the instrument relevant to the objectives of the study?				
2.5. RESEARCH PERSPECTIVE – RESEARCH TYPECLINICAL				
In what perspective is the research located? What type of research is this? (Check the corresponding box(es) Observational research (descriptive. analytical? Interventional research Therapeutic trial, vaccine trial, evaluation of biomedical material Research? National, regional or institutional study? Multi country search?				
3. ETHICAL CONSIDERATIONS				
Is a chapter entitled "ethical consideration" included in the protocol?				
If yes, does this chapter include the following elements: the ethical justification of the protocol, the identification of ethical issues or risks, the measures taken to minimize risks, the texts applicable regulatory requirements, the conclusions of previous ethical evaluations, the plan for disseminating the results and the plan improving access of				

target populations to research conclusions?				
Is the identification of ethical issues appropriate?				
RISK/BENEFIT ANALYSIS				
What are the direct and indirect benefits of the study?				
Does the project include an inventory of expected benefits in terms of improved efficiency and safety?				
Does the project include an inventory of the expected benefits in terms of improving access to care or in terms of the provision of care?				
Was this inventory done on an individual level (physical, physiological, psychological, emotional, dignity and rights?				
Was this inventory made on a social level (disturbances of family, social and cultural norms and organizations?				
)? Was this inventory carried out on an environmental level (could have an impact on individuals or society?				
Are the benefits reasonable in relation to the risks (is there an alternative or procedure that can improve the risk/benefit ratio?				
5. RESPECT FOR CONFIDENTIALITY				
Is confidentiality ensured?				
Data collection tools, procedures for collecting, analyzing and presenting results designed to avoid identification of participants?				

Is access to databases restricted to the research team?				
Is the coding system appropriate (in case there is a code register)?				
6. NOTICE D'INFORMATION				
Is there an information leaflet in English and French?				
Does the information notice include the following information?				
Project title names and full address of the principal investigator				
study procedures (mode and time of administration of the questionnaire. sample collection, sampling, randomization, data analysis, duration of data collection and monitoring, frequency of visits):a description of the benefits and risks of the study.an estimate of the person's time involved: the possibility for the participant to refuse and withdraw their consent at any time without incurring a sanction: possible compensation and justification of the amount; the address of the people to contact in the event of concerns or the occurrence of problems (email address, geographic address, mobile and landline telephone number of the principal investigator and the competent ethics committee)				

Is it expected that a copy of the notice will be given to the participant				
6.1. CONSENT /ASSENT				
Does the study provide for self-collection of consent or for a team member to read and explain the consent documents to the participant?				
Are all Consent/assent documents available in French and English?				
Do these documents contain the following information: the title of the project; the acceptance sentence; space for the participant's name; the method of consent (verbal or written)				
space for the signature of the participant and any witnesses; the date of signature by the participant/legal representative; the name of the person obtaining consent				
the possibility of accepting or refusing consent for the use of their data or samples for subsequent studies.				
Are the language and language used on the consent/assent documents simple and understandable for the participant and/or their representative?				

Is the translation of consent documents into local dialects clearly stated?				
Is the procedure for obtaining consent clearly described in the protocol?				
Does this procedure expose the participant to: subordination, induction, pressure and/or exploitation?				
Does the study provide for self-collection of consent or for a team member to read and explain the consent documents to the participant?				
Does the consent procedure allow the participant to ask questions for clarification before participating?				
Does the place where consent is collected respect the confidentiality and comfort of the participant?				
If the study requires verbal consent from the participant/legal representative, does the procedure provide for the validation of consent, in particular by the signing of the form by a witness and by the person who collects the consent?				
? Is there documentation of consent/assent tailored to each type of study participant and data source?				
Direct data sources: adults (21 years and over), children (children aged 12), adolescents (12 to 20 years), vulnerable people, participants in emergency situations?				

Source of indirect data (patient files, database, serum library or biobank)?				
Community or group?				
The form includes: date of signature by participant/legal representative, name of the participant, name of the person collecting consent, commitment statement from the participant?				
Does the procedure provide for the possibility of accepting or refusing consent for the use of their data or samples for subsequent studies?				
In the event that the participant is a minor under the age of 12. Is the information notice containing general information intended to obtain consent offered to the parent or guardian?				
Is a parental consent form offered?				
If the participant is a minor aged 12 to 20: is an assent form offered to the child?				
Is a parental consent form offered?				
In the case of the consent of an emancipated minor (Individual <21 years old, married minor, and by legally valid decision of the parents from the age of fifteen): does the study plan to apply the same consent procedure as in 'adult?				

In the case of the consent of vulnerable groups/people (any person presenting a health, physical, social, and/or economic situation which does not allow them to assume their autonomy and/or to freely give consent: children, pregnant women, newborns, fetuses, young homeless people, people with mental/psychiatric disorders, economically or educationally disadvantaged, marginalized, people in terminal illness, prisoners, etc.), except for information notice containing general information, does the study provide a consent form allowing the legal guardian to give permission for the person to participate in the study?				
Does the study give vulnerable participants who have regained their autonomy the opportunity to give their consent?				
In the case of consent from people in medical emergencies (foreseeable emergencies - people with unforeseeable emergencies are classified among vulnerable groups), is the collection of consent planned before the emergency occurs?				
EXEMPTION CASE/ WAIVER OF INFORMED CONSENT. Is the data source indirect with the following situations:				
Inability to access the participant(s); minor risk of violation of autonomy and confidentiality; qualitative				

observational and ecological studies where the target is a group and not an individual.				
Is the use of health registries/databases authorized by the institutions that host them?				
Does the confidentiality procedure used protect access to or use of personal information in the registers/databases?				
Does the waiver of informed consent affect the rights and well-being of research participants?				
Does the study involve biological samples and/or personal data obtained with the consent of the participants for subsequent use?				
PARTNERSHIP COLLABORATIVE TO IMPROVE ACCESS TO RESEARCH RESULTS				
7.1. DATA SHARING AGREEMENT (DSA)				
The data comes from and/or is used by several institutions?				
Is there a data sharing agreement signed by all stakeholders?				
Does the study plan for the collected database to be hosted in Cameroon?				
Do procedures for securing the data collected exist?				
Order of authors in publications				
is it specified in the DSA?				
Is this order of authors in publications fair ?				
Is access to the collected database equitable (access by local investigators to the database of all sites of a				

multi-country or multicenter study)?				
7.2 MATERIAL TRANSFER AGREEMENT (MTA OR MATERIAL TRANSFER AGREEMENT)				
Is there an MTA including at least the following information				
The nature, quantity and frequency of sample transfer?				
The description of the samples in terms of source, volume, method and duration of storage and transport?				
The names and addresses of the sender and his institution?				
The mode of destruction?				
The conditions for processing derivative products?				
The conditions for sharing with other laboratories?				
Equity in the sharing of profits generated by products derived from the biological material transferred (intellectual property, operating patent and marketing)?				
References to the law applicable to sample safety and the person responsible?				
Is the MTA agreement document signed by the following officials: the shipper, the recipient and the sample security officer?				
7.1. GUARANTEE FOR SHARING OF RESULTS OF RESEARCH (multi-country and multi-institution studies) Is there a plan for collaboration between stakeholders (government, sponsor health authorities and community) guaranteeing population access to research results on				

the financial, technological (technical platform) and training of human resources levels?				
Is there a plan to strengthen the capacities of Cameroonian co-investigators?				
Is there a technology and skills transfer plan?				
CONSIDERATIONS				
8.1 SKILLS OF THE RESEARCHER(S) AND INVOLVEMENT IN THE RESEARCH				
8.1. Does the principal investigator have the skills and experience required to conduct the research?				
Does he present a profile that meets the research requirements?				
Is his responsibility assumed within the framework of the study?				
Is there a presentation from the research team?				
Are the CVs of team members included in the project?				
Is the research team competent to meet the research objectives?				
8.3. CHRONOGRAM OF ACTIVITIES				
Is a timetable of activities attached to the protocol?				
Is the planning for data collection and analysis described and consistent?				
Does the duration of the research allow the expected results to be obtained in order to resolve the study problem?				

8.4 8.4. BUDGET				
Is there a budget attached to the project?				
Is this budget detailed?				
Is this budget consistent with the duration of the study?				
Is this budget sufficient to carry out the work?				
Does this budget provide for the restitution of the results of the study?				
8.5. AGREEMENT IN PRINCIPLE OF RESEARCH SITES				
Is there an agreement in principle between research sites bearing the signature of their manager?				
9. FINAL PROVISIONS				
9.1. AREA OF COMPETENCE OF THE EVALUATOR (specify)				
9.2 Conclusion				
9.3. GENERAL RECOMMENDATIONS				

Appendix 2: Ethical monitoring grid for a research project				
Section 1: General				
Date of field visit				
Names of the individuals encountered				
Names of supervisors				
Title of the study				
Type of study				
Promoter				
Principal investigator Co-investigators				
Expected duration of the study				
Source of funding for the study				
Has participant recruitment started? If no, give details.				
How many participants have been recruited to date?				
Is participant recruitment continuing?				
How many participants remain to be recruited?				
Are there any drop outs from the study? If yes, give details.				
Do recruited participants always receive visits/care? If yes. give details.				
(need for the study, expected benefits, risks incurred): and in clear, understandable language adapted to the level of understanding of the population targeted by the research?				
3.6. Is the subject informed about what will happen to the data collected, who will have access to it and when it will be destroyed?				

3.7. Does the subject have the opportunity to ask questions to better clarify their questions?				
3.8. Does the subject have time to think about their acceptance (are they given time between transmitting the information and signing the consent form)?				
3.9. Is the information transmitted to the participant by a member of the research team?				
3.10. Is the information transmitted to the participant by a person independent of the research team?				
3.11. Do individuals decide for themselves to participate in the study independently of any consideration of costs and benefits?				
3.12. Are subjects subjected to threats/pressure/manipulation to obtain acceptance?				
3.13. Do subjects receive an unwarranted and inappropriate reward/promise for their participation?				
3.14. In the case of research involving more than one data collection phase, is participants' consent renewed throughout participation in the study?				
3.15. Are subjects informed of the voluntary nature of their participation?				
3.16. Are participants free to reconsider their decision at any time?				

3.17. For research involving people incapable of giving voluntary consent, are the following conditions respected:				
- consent/authorization of the legal representative?				
- consent for children or fragile and vulnerable people?				
3.18. all subjects participating in the study signed informed consent forms?				
Section 4: Risk and benefit analysis	Yes	No	N/A	Observation
4.1. Are subjects informed of all anticipated risks, as well as the measures that will be taken to reduce them?				
4.2. Are subjects informed of the anticipated advantages or benefits of the study?				
4.3. Are the safety and well-being of participants ensured?				
4.4. Is the use/collection of any biological material justified?				
4.5. Does current scientific knowledge				
4.6. Were there any unanticipated problems related to the study?				
4.7. Do subjects receive compensation, of a justified nature and value, due to their participation in the research?	*			
4.8. Is the privacy of participants respected?				
Section 5: Selection of topics	**Yes**	**No**	**N/A**	**Observations**
5.1. Is the choice of research				
5.2. Does the research involve minors?				

5.3. Does the research involve vulnerable subjects/groups (any person with a health, physical, social, and/or economic situation which does not allow them to assume their autonomy and/or freely give consent: children, pregnant women, newborns, fetuses, homeless young people, people with mental/psychiatric disorders, economically or educationally disadvantaged, marginalized, people in terminal illness, prisoners etc.)?				
5.4. Are the people or groups recruited likely to benefit from the research?				
5.5. Do people in vulnerable situations enjoy fair treatment?				
5.6. Are the inclusion criteria justified by the research question?				
6.2. Are consent forms, physical data, digital data, databases, biobanks kept in such a way as to protect the confidentiality of participants?				
6.3. Is access to data restricted to duly authorized persons?				
Section 7: Adherence to the research protocol	**Yes**	**No**	**N/A**	**Observations**
7.1. Did the study sponsor commission internal/external audits of the research project?				
7.2. Has a designated manager or monitoring				

committee with relevant expertise been appointed to oversee the study?				
7.3 Is there a data management system for the study?				
Is a database available?				
Do procedures exist for data integrity?				
7.5 Is there a system for archiving all essential documents and activities carried out within the study? If yes, which ones? -Monitoring reports - Meeting reports - Progress reports - Quality assurance reports				
7.6 Is there a system for reporting adverse events to the ethics committee and regulatory authority? If yes, are the adverse event notification forms available?				
7.7 Were there any adverse events related to the study? If What is the number of adverse events notified to the ethics committee and the regulatory authority? Are serious adverse event reports available? Is there consistency between the data recorded in the case report forms and that contained in the notification forms? Are the responses from the ethics committee and the regulatory authority available?				
7.8. Are sample collections, medical imaging studies, or other tests performed as described in the approved research protocol?				

7.9. Have there been any changes to the protocol approved by the ethics committee and regulatory authorities? Is written proof of acceptance of protocol amendments by the ethics committee available; Have the protocol amendments approved by the ethics committee been notified to the regulatory authorities?				
Section 8: Study Site				
8.1. Does the site have the necessary equipment and infrastructure for this study?				
-Is there a space dedicated to physical examinations and biological samples?				
-Are the necessary equipment and instruments available and appropriate?				
-Are hospital beds available?				
- Is a well-equipped emergency area with medications to administer in an emergency available?				
- Are medications to be administered in an emergency stored according to their shelf life?				
8.2. Are laboratories adequately equipped to perform the required tests?				
- - Are the spaces intended for reception, preparation, and storage until the time of sample analysis appropriate?				
- - Is the qualification and calibration status of laboratory equipment and instruments valid for use for sample analysis?				

- Have contracts been signed for subcontracting?				
- Are the sample storage and handling procedures followed?				
- Are participant samples traceable?				
8.3. Does the site meet hygiene and biosecurity standards?				
- Availability of toilets and washing facilities;				
Presence of waste disposal system;				
Section 9: Research staff	Yes	No		N/A
9.1. Did research staff receive training before the start of the study?				
9.2. Have all staff responsibilities (sponsor, investigators, other staff) and organizational chart been described?				
If applicable, is evidence available of the delegation of tasks to staff based on their qualifications and experience?				
9.3. Does the staff profile meet the requirements?				
- Are documents relating to staff qualifications, including contractual staff, available?				
- Are staff experience and training appropriate?				
Is there enough staff to conduct the study?				

Date de la descente sur le terrain					
Noms des personnes rencontrees					
Nom et address du site					
Noms des supervisieurs					
Ite m		No	Indicator	Reponse	Observati on
1	Availability of resources for the protection of research participants during the implementati on of research projects carried out in the organization.	1.1	Proportion of researchers with at least one completed degree (PhD) covering their research field.		
		1.2	Proportion of researchers who have received at least 40 hours of training in research ethics for health.		
		1.3	Proportion of researchers who have received training covering the specific ethical aspects of their research field.		
		1.4	Proportion of researchers with regulatory documents on the protection of research participants.		

		1.5	Proportion of research projects conducted by the organization that incorporate tools ensuring participant protection.		
2	Evaluation ethique et adminsitrativ e des projects de recherche mis en oeuvre dans l'institution de recherche	2.1	Proportion of implemented research projects having obtained ethical clearance issued by a competent ethics committee		
		2.2	Proportion of research projects having benefited from an Administrative Research Authorization (AAR) issued by the Ministry of Health		
		2.3	Proportion of research projects with exchange of biological material between institutions and with a Material Transfer Agreement		
		2.4	Proportion of research projects with data exchange between institutions and with a Data Sharing Agreement		
		2.5	Proportion of clinical trials registered in a public local and/or international database (Pan-African Clinical		

			Trials Registry at minimum)		
		2.6	Proportion of completed research projects that submitted a final report to the relevant ethics committee		
3	Disseminatio n of results	3.1	Proportion of completed research projects whose results have been disseminated to the target communities of the research		
		3.2	Proportion of completed research projects whose results have been disseminated to decision-makers/health authori ties		
		3.3	Proportions of projects completed research including results were presented at scientific conferences national		
		3.4	Proportions of projects completed research including results are accessible in at least one newspaper scientist / library free access		

Appendix 4: Guidelines for the monitoring of the Ethics committee by DROS

No	Item	No	Indicator	Response	Observation
1	Access to ethical evaluation	1.1	Proportion of regions covered by an Ethics Committee for Human Health Research (CERSH)		
		1.2	Proportion of regions covered by a CERSH		
		1.3	Proportion of CERSH set up according to regulations		
		1.4	Proportion of CERSH having office space for reception and evaluation of protocols1.6 Proportion of CERSH having a public meeting calendar		
		1.5	Proportion of CERSH having a public meeting calendar		
		1.6	Proportion of CERSH having respected the public meeting schedule over the last 12 months		
		1.7	Proportion of CERSH with standardized operating procedures and evaluation of protocols		
		1.8	Proportions of first and second category hospitals with a CERSH		

		1.9	Proportion of CERSH having the convenient structure to that recommended by the regulations		
		1.10	Proportion of CERSH having a composition convenient to that recommended by regulations		
		1.11	Proportion of CERSH constituted which are functional		
2	Selection and training of members		2.1. Proportion of members selected according to guidelines		
		2.2	Proportion of members trained in ethical assessment		
		2.3	Proportion of members accumulating 40 hour s training and research ethics over the last 36 months		
		2.4	Proportion of members trained in good clinical practices over the last 36 months		
		2.5	Proportion of members trained in clinical drug trial evaluation in the last 36 months		
		2.6	Proportion of members trained in vaccine clinical trial evaluation in the last 36 months		

		2.7	Proportion of members trained in clinical trial evaluation of traditional medicines in the last 36 months		
3	Scheduling and holding meetings		3.1. Proportion of CERSH with an annual calendar		
		3.2	Proportion of CERSH having an annual calendar made public		
		3.3	Proportion of CERSH holding at least 80% of meetings to date planned		
		3.5	Proportion of CERSH holding at least 80% of meetings with at least 80% of its members		
		3.6	Proportion of CERSH meetings holding at least 80% of meetings convened properly		
		3.7	Proportion of CERSH with standardized operating procedures for ethical evaluation		
		3.8	Proportion of CERSH using the ethical and administrative evaluation guide for human health research protocols from the Ministry of Health for ethical evaluation		

		3.9	Proportion of evaluation meetings with prior declaration of conflict-of-interest		
		3.10	Proportion of evaluation meeting with prior confidentiality declaration		
		3.11	Number of research protocols received in the last 12 months		
		3.12	Number of research protocols eligible for ethics review over the past 12 months		
		3.12	Proportion of research protocols forwarded for evaluation according to the procedure provided for by current directives		
		3.13	Proportion of protocols evaluated within the time limits set by the directives current applicable and benefiting from ethical clearance		
		3.14	Proportion of protocols evaluated within the time limits set by the directives		
		3.16	Proportion of ethics evaluation meetings with report		
		3.17	Proportion of ethics evaluation meetings archived in soft and hard		

		3.18	Proportion of ethics evaluation meetings with report shared with the stakeholders provided for in the applicable directives		
		3.19	Proportion of evaluation meeting ethics with report indicating the method of decision-making for each protocol evaluated.		
4	Financing of Ethics committees	4.1	Total amount of funding received in the last 12 months		
		4.2	Proportion of financing lines corresponding to those provided for by regulations		
		4.3	Percentage of the amount of funding received corresponding to the amounts provided for by regulation		
		4.4	Proportion of payments made to CERSH members according to current regulations		
5	Archiving	5.1	Proportion of CERSH receiving ethical clearance request files electronically		
		5.2	Proportion of CERSH receiving ethical clearance request files electronically		

		5.3	Proportion of CERSH making an electronic archiving of ethical clearance request files		
		5.4	Proportion of CERSH having an electronic database of ethical clearance request files received		
6	Communication		6.1. Proportion of CERSH reachable by dedicated telephone contact		
		6.2	Proportion of CERSH reachable by dedicated email contact		
		6.3	Proportion of CERSH reachable by dedicated post office box		
		6.4	Proportion of CERSH with a website		
		6.5	Proportion of CERSH having a public information web page		
		6.6	Proportion of CERSH having an interface for submitting ethical clearance request files online		

Appendix 5: Review grid for the functioning of the Ethics Committees for Human Health Research in Cameroon (CERSH)

Date of field visit				
Name and address of the ethics committee				
Names of people met				
Supervisor Names				
No.	Variable	**Modality**	**Response**	**Comments**
1	Target of assessment	FOSA category 1= a FOSA category 2= b FOSA category 3= c Training institute= d Research		
2	Type of committee	National = a Regional = b Institutional = c		
3	Date of establishment of the committee	Date: DD/MM/YYYY		
4	Can you give us a copy of the minutes/official document establishing the	Yes=l, no = 2,		
5	Date of last committee renewal		Date: DD/MM/YYYY	
6	Can you give us a copy of the renewal report?	Yes = 1. no = 2,		
7	Can you give us a copy of the decision to establish the	Yes=l, no = 2,		

8,	Signing authority of the finding decision	Minister of Public		
		Chairman of the board of directors = 4 Other = 5		
9	Total number of members in the committee	Number		
10	Total number of committee members currently attending meetings	Number		
11	Number of women on the committee	Number		
12	Number of doctors on the committee	Number		
13	Number of statisticians on the committee	Number		
14	Number of community representative	Number		
15	Number of members not working * in the structure which hosts the committee	Number		

16	Number of committee members who have received at least one ethical evaluation training	Number		
17	Number of committee members having received at least 40 hours of training in ethical evaluation	Number		
18	Can you give us a copy of the committee meeting schedule for last year?	Yes= 1, no - 2,		
19	Is said calendar made public?	Yes=l, no = 2,		
20	If the calendar is public, from what source	Poster = a		
	can we check it?	Website = b		
21	How many meetings were planned	Number		
	over the last 12 months?			

22	How many meetings were held in the last 12 months?	Number		
23	Number of meetings held by the committee in the last 12 months with minutes	Number		
24	Does the committee have a physical address for - submitting ethical review requests?	Yes=L no = 2,		
25	Does the committee have a telephone line	Yes=l, no = 2,		
26	Does the committee have an email address?	Yes=l, no = 2,		
27	Does the committee have a website?	Yes—], no = 2.		
28	Does the committee have a post office box?	Yes=l, no=2.		

29	Does the committee have a bank account for depositing protocol evaluation fees?	Yes=l, no = 2,		
30	Does the committee have a meeting room for protocol evaluation?	Yes=L no = 2,		
31	Does the committee have a secretariat?	Yes=l, no = 2,		
32	Does the committee have a computer?	Yes^l, no = 2,		
33	Does the committee have a printer?	Or NI, no - 2,		
34	Does the committee have a document archiving system?	Yes=l, no=2.		
35	Does the committee have standard operating procedures for	Yes = 1, no = 2,		
	operation and ethical evaluation?	•		

36	Does the committee have the parts list? required for ethical assessment	Yes=l, no = 2,		
37	Number of ethics review requests received over the last 12	Number		
38.	Number of ethics review requests received over the last 12 months deemed admissible			
39.	Number of ethics review requests received over the last 12 months deemed admissible and examined			
40.	Number of ethics review requests received in the last 12 months deemed admissible, examined with ethics clearance issued			
41.	Number of ethics review requests received in the last 12 months deemed admissible, examined and rejected			
42.	Average duration of evaluation of a protocol			

43.	43. Number of research projects for phase 1 clinical trials reviewed in the last 12 months			
44.	Number of research projects for phase 2 clinical trials reviewed in the last 12 months			
45.	Number of research projects for phase 3 clinical trials reviewed in the last 12 months			
46.	Number of site visits, for evaluation before approval of a phase 1 clinical trial, conducted			
47.	Number of phase 1 clinical trials followed after approval in the last 12 months *			
48.	Number of research projects followed after approval during the last 12 months			
49.	Number of annual reports on the functioning of the committee available			

Appendix 6: Conflict of interest declaration form by representatives of research institutions

A. CONFLICT OF INTEREST

1. The research team is a collaborative entity between members of various disciplines and institutions who are required to declare any conflict of interest.
2. A conflict of interest occurs when a member has an interest that could distort their opinion, endanger the study participant and damage the reputation of the research team. Conflicts of interest may include personal, professional, financial or other interests.
3. It is the responsibility of each member of the research team to declare any conflict-of-interest situation that may arise, whether real or apparent. Potential conflicts that develop during the duration of the study should also be disclosed. However, the appearance of a conflict of interest must be avoided as much as possible.

The following elements should be considered when assessing whether or not a conflict of interest exists:

• Employment: current employment or employment dating from the last 12 months (employee, consultant or other recipient of fees, member of the board of directors or advisory committee) with an institution or organization that is directly impacted by the results of the research or who could benefit financially from it.

• Financial benefit: a person who has received or could receive a direct financial benefit in any amount other than their remuneration, from a person, institution or organization involved in activities

similar to those for which he is requested within the framework of the planned research is in a situation of conflict of interest.

• Intellectual property: Intellectual property, such as copyrights, patents related to activities similar to those of the intended research, may be considered a conflict of interest.

• Relatives or associates: A conflict of interest may exist if a close relative or professional associate of a member of the research team receives or could receive any benefits other than those generated by said research.

• Long-standing disagreements: A conflict of interest may exist when one member of the research team has had long-standing scientific or personal

differences with another, which may compromise the implementation of the research.

B. COMMITMENT REGARDING CONFLICTS OF INTEREST

NAME: ----------

Main employer: ----------

I have read the attached conflict of interest form.

I hereby certify that I have no conflict of interest.

Signature: Date:

Appendix 7: Confidentiality Undertaking Form

A. CONFIDENTIALITY

1. Documents and data made available by the research team and which are not in the public domain, as well as discussions resulting from the data analysis, are strictly confidential and should not be disclosed or discussed with anyone, who is not a member of the research team.

2. Confidential information obtained as a member of the research team cannot be used by said member for his or her personal benefit or for the benefit of his or her family.

B. PRIVACY COMMITMENT

NAME:

Main employer:

I have read the attached privacy form:

I fully understand the nature of the confidentiality process and undertake not to disclose confidential documents, data and discussions associated with the work of this research.

Signature:

Date:

Appendix 8: Outline of the agreement for the transfer of material for research purposes involving human beings

Within the scope of the missions of Institution X, a public scientific and technological establishment, it agrees to grant non-exclusive rights of use for non-commercial research purposes to third parties belonging to the scientific community, as described below.

Specifically, Institution X gives its consent for the transfer of the described biological material, particularly the researcher responsible for said biological material (hereinafter referred to as the "Scientist"), to the benefit of the institution and the applicant (hereinafter collectively referred to as the "RECIPIENTS" and described below), subject to the prior acceptance by the recipient of the terms and conditions of this agreement (hereinafter "Agreement").

Institution Employer of the applicant	Nom
	Physical Address ----------
	Tel;
	Fax;
	Email;
Requester of the original material	Nom
	Function
	Physical Address
	Tel;
	Fax

	Email
Premises from which the samples will be prepared for shipment	Nom
	Address
	Tel
	Fax
	Email
Origin of the material and requested quantity.	Source of the material (human, animal, bacteria?).
	Description of the specimen (tissue/cells, volume/weight per unit).
	Total quantity of samples to be shipped.
	Periodicity
Institution employing the beneficiary.	Nom
	Function
	Physical Address ----------
	Tel:
The beneficiary researcher.	Nom
	Physical Address
	Tel
	Fax

	Email
Address of the premises where the research will be conducted.	Nom
	Physical Address
	Tel
	Fax
	Email: ----------
Research to be undertaken by the applicant using the original material (may be supplemented by an Annex, which will be an integral part of the agreement).	

DEFINITIONS

Commercial Applications: Sale, rental, license, or other transfer of the Hardware or Modifications to a for-profit organization. Commercial Applications also include the use of the Material or Modifications by any organization, including the BENEFICIARY, for the performance of research contracts, the querying of compound libraries, the production or manufacturing of commercial products or the conduct of research activities resulting in sales, rentals, licenses or transfers of the Materials or Modifications to a for-profit organization.

(ii) **Descendant**: Unmodified descendants of the Material such as virus from virus, cell from cell, or organism from organism.

(iii) Unmodified Derivatives: Substances created by the BENEFICIARY that constitute an unmodified functional subunit, or product expressed by the Original Material. Some examples: subclones of unmodified cell lines, by-products expressed by DNA/RNA, or monoclonal antibodies secreted by cell line hybridoma.

(iv) Material: Original Material, Unmodified Descendants and Derivative Materials. The Material does not include: a) Modifications, b) other substances created by the BENEFICIARY using the Material and which are neither Modifications, nor Descendants, nor unmodified Derivatives.

(v)Original Material: The research material transferred by the supplier accompanied by related information.

(vi) Modifications : Substances created by the BENEFICIARY that contain/incorporate the Material.

CONDITIONS OF AGREEMENT

1) **SUBJEC**T

1.1. The BENEFICIARY acknowledges that this Agreement aims to encourage scientific cooperation aimed at the development of applications of the Original Material as well as the exchange of technical data.

1.2. The Supplier Institute agrees to provide the BENEFICIARY with the Original Material under the conditions provided for in this Agreement.

2) SUPPLY OF ORIGINAL MATERIAL

2.1. The Scientist will send the Original Material to the Address for the attention of the Applicant.

2.2. The supplier and the Scientist cannot be held responsible for any transport damage. In the event that the Original Material does not arrive or arrives at the Address in such conditions that it is unusable, the Scientist will return the Original Material to the Address for the attention of the Requester.

2.3. The costs of shipping the Original Material will be included, where applicable, in the financial conditions provided for in this Agreement.

3) INFORMED CONSENT

If material collected is part of human biological tissue or is identifiable, research participants must be informed of the purpose, the justification for the transfer, the objectives of the research that will be carried out, the precise procedures, risks, quantities, duration and address. places of conservation and transfer and the people who are responsible for them. They must at the same time be informed of their freedom to refuse the sample and whether it is possible for them to decide to withdraw their sample at any time without risk. They must also be informed of amendments to the objectives of the research protocol and of amendments that could result in additional risk before implementation.

4) APPROVAL OF THE ETHICS COMMITTEE AND COMPETENT AUTHORITIES

If research involves the collection and transfer of human biological material from one institution to another, this must be specified in the research protocol and submitted for evaluation to the ethics committee and the Division of Operational Research in Health. (DROS) at the same time as the “Material Sharing Agreement (MTA)”

All subsequent modifications to the protocol and/or the MTA must first be approved by the ethics committee and the DROS.

5) USE

5.1. The BENEFICIARY undertakes to ensure that the Material:

(a) is used solely for the purposes of the Research described in this Agreement;

(b) is not distributed or transmitted to a third party for any purpose whatsoever;

(c) born. is not used for other research without the advice of the SUPPLIER;

(d) is used in accordance with international and Cameroonian regulations on the transfer of biological material

(e) is used exclusively at the BENEFICIARY and by the scientists working in the BENEFICIARY's laboratory or under its direct responsibility;

(f) is accessible to the SUPPLIER even if the researcher currently responsible for the BENEFICIARY institute is no longer under contract with his employer.

5.2. The BENEFICIARY has the right, without any restriction, to distribute the substances he created using the Original Material, to the extent that these substances are not Descendants, Unmodified Derivatives or Modifications.

5.3. UNDER NO CIRCUMSTANCES may the BENEFICIARY provide Modifications for Commercial applications without the prior written consent of the SUPPLIER.

5.4. The BENEFICIARY acknowledges that the Material is or may be the subject of a patent application.

Except as provided in this Agreement, no express or implied license or other right is granted to RECIPIENT in any patents, patent applications, trade secrets or other proprietary rights of SUPPLIER.

5.5. The BENEFICIARY acknowledges that this Agreement cannot be interpreted as constituting a license for its benefit or a commitment or obligation to enter into any subsequent agreement.

6) PROPERTY

6.1. The providing institution retains ownership of the Material, including the Material contained or incorporated in the Modifications.

In addition, the supplier institute retains all rights which may be conferred upon it by intellectual property laws on inventions, particularly patentable ones, which may result from the BENEFICIARY's use of the Original Material.

6.2. The Modifications carried out jointly by the supplier institute and the BENEFICIARY, or by the BENEFICIARY alone, are the co-ownership of the supplier institute and the BENEFICIARY.

6.3. The BENEFICIARY retains ownership of the substances that he or she created alone using the Material or Modifications, with the exception of Descendants, Unmodified Derivatives or Modifications (i.e. which do not contain the Material of 'Origin, Descendants or Unmodified Derivative). A co-ownership agreement will be negotiated for those of these substances which result from joint collaborative efforts between the providing institute and the BENEFICIARY.

6.4. With the exception of the provisions of 6.1, 6.2 and 6.3 above, ownership of any Research results, whether patentable or not, will depend on the inventive contribution of the participants in obtaining them.

In the event that all or part of the results of the Research could be protected by a new patent application designating one or more inventors of the supplier institute and the BENEFICIARY, the parties will consult in order to define the terms of filing such an application patent, as well as its operating conditions.

6.5. The BENEFICIARY undertakes not to file without written consent of the supplying institute, in its name or that of a third party, in any country whatsoever, an application for patents or intellectual property rights (copyright, trademark, etc.) relating to the Material.

6.6. The BENEFICIARY undertakes to provide the supplier institute, free of charge and as soon as possible, with the Modifications resulting from the use of the Equipment.

7**) PUBLICATION - CONFIDENTIALITY**

7.1. This Agreement shall not prevent or delay the publication of research results resulting from the use of the Material or its Modifications. The BENEFICIARY will submit to the provider institute a copy of any draft publication.

7.2. In accordance with scientific practices, the contribution of the agents of the supplier institute who provided the Material or of their possible collaborators, will be expressly mentioned in any oral or written intervention relating to the results of use of the Material, either by thanks, or as co-authors, according to the most appropriate formula. The origin of the

Material as well as the indication of any corresponding patents must appear in these disclosures.

7.4. The BENEFICIARY undertakes to respect and maintain the strictly confidential nature of any information identified as confidential received from the Supplier Laboratory.

The BENEFICIARY guarantees respect for the provider institute by its staff and or any person attached in any capacity whatsoever to its service of the confidential nature of said confidential information.

The BENEFICIARY undertakes to use confidential information only within the framework of this Agreement.

The provisions of 7.4 take effect from the date of entry into force of this Agreement.

8) FINANCIAL CONDITIONS

The Material is transmitted:

□ Free of charge

□ In return for the sum of……..FCFA for the reimbursement of preparation and shipping costs. In this case, the supplier institute will send an invoice, which will mention the payment terms (deadline, account), to the institution at the address mentioned on the first page hereof for the attention of the Applicant or any other person expressly designated. (Check the corresponding box)

9) GUARANTEES

9.1. The BENEFICIARY accepts the Original Material as is and acknowledges that it is of an experimental nature and that it must therefore be used with caution and caution, since not all of its characteristics are known and that it may have dangerous properties. THE SUPPLIER INSTITUTE DOES NOT MAKE ANY REPRESENTATION OR OFFER ANY WARRANTY, EXPRESS OR IMPLIED, OF ANY KIND WHATSOEVER. ALSO, NO EXPRESS OR IMPLIED WARRANTY IS GRANTED BY THE SUPPLIER INSTITUTE AS TO THE COMMERCIAL CAPACITY OF THE MATERIAL OR ITS SUITABILITY FOR A PARTICULAR PURPOSE OR AGAINST ANY INFRINGEMENT. THE SUPPLIER INSTITUTE, its directors, employees or agents assume no responsibility and take no action with regard to the Material or its use by the BENEFICIARY. The latter will defend, indemnify and hold harmless the provider institute, its directors, employees and agents from any damages, claims or liability that may result from the use of the Material or information relating thereto.

9.2. The provider institute does not guarantee that the use of the Material does not infringe the intellectual property rights of third parties.

10)TERMS OF AGREEMENT

10.1. This Agreement shall enter into force on the later of the dates set forth on the signature page and shall terminate on the earlier of: (a) 2 years from the date of signing hereof, or (b) upon completion of the research work carried out by the BENEFICIARY using the Material or (c) within 30 days following the sending by one of the parties of a written notice of termination, provided that:

-If the Agreement is terminated under (a) or (b), the BENEFICIARY will cease use of the Materials and, as directed by the providing institution, return or destroy any remaining Materials. The BENEFICIARY may, at its discretion, destroy the Modifications, or remain bound by them. terms of the Agreement with respect to Modifications.

-If the provider institute terminates the Agreement under (c) for reasons other than non-compliance with the terms of the Agreement or for causes such as knowledge of an imminent health risk or infringement of a patent, The supplier institute may, at the request of the BENEFICIARY, grant an extension of the Agreement for a maximum period of one year, in order to allow the completion of the work in progress.

10.2. At the end of this period or on the effective expiry date, the BENEFICIARY will stop using the Material and, according to the instructions of the providing institute, will return or destroy any remaining Material. The BENEFICIARY may, at its discretion, destroy the Modifications, or remain bound by the terms of the Agreement with respect to the Modifications.

11) MISCELLANEOUS

11.1. This Agreement is governed by the laws of Cameroon and that of the country of the BENEFICIARY's institution. The Cameroonian courts have jurisdiction over any dispute relating to the interpretation or execution of the Agreement that the parties have not been able to resolve amicably.

11.2. This Agreement constitutes the entire Agreement between the provider institute and the BENEFICIARY in relation to its subject matter and replaces and cancels all declarations, negotiations, commitments, oral or written communications, acceptances, understandings and prior agreements, relating to the provisions to which this Agreement applies or provides for, whether or not incorporated. If any provision of this Agreement is found to be unenforceable for any reason the relevant provisions will be modified to make them enforceable and such modifications will not affect the applicability (i) of such provision in other circumstances or (ii) the other clauses in all circumstances.

Supplier Institution Signature Name Title Date	Recipient Signature Signature of a duly authorized representative to bind the institution/company by their signature. Name Title Date
Read and approved by the scientist. Signature Title Date	Read and approved by the applicant. Signature Title Date

Appendix 9: Inspection checklist for Clinical Trials

Section 1: Generalities	
Date of inspection	
Name and address of the clinical site.	
Name and address of the laboratory (clinical, analytical).	
Names of the individuals met.	
Names of the inspectors.	
Reference (Annex)	
Title of the study	
Protocol identification number, Protocol version	
Drug/Product under study	
Registration number in the Pan-African Clinical Trials Registry	
Trial phase	
Sponsor	
Principal investigator	
Co-investigator	
Stage of study being inspected (start, during, end)	

Section 2 : Data management system	**Yes**	**No**	**Partial**	**Not applicable**	**Observations**
2.1. The study received inspections					
2.2 Organization of internal/external audits as part of the study					
2.3. Is there a data management system for the study? Database name and/or database management tools; data integrity policy; The data and results were examined and considered exhaustive, complete and reliable (opinion of the inspectors).					
2.4. Is there documentation and archives on all the activities carried out as part of the study? Policies, procedures and responsibilities have been documented and followed; Documents relating to the investigational					

medical product, including the export license or import authorization were available; The supporting documents for purchases, the shipping slip, the storage conditions during transport of the experimental products were available; The on-site receipt slip, the analysis certificates, the stock sheet and the distribution register for the experimental products were available; Management of deviations and violations, risk management principles, and corrective and preventive actions; Qualification and training files for employees and contract staff were available; Protection against risks of degradation by fire, water, humidity and deterioration					
2.5. Are there any reports on					

the activities carried out as part of the study? which? The reports of the manager follow up Meeting reports Quality assurance reports covering the review of data and information					
Strengths:					
Weak points:					
3.1. Have there been any changes to the protocol approved by the ethics committee and regulatory authorities? Evidence of written acceptance of the protocol and its annexes or amendments by all researchers was available; Notification of protocol modifications to authorities; The list of changes. Strong points Weak points					
Section 4: Ethical and regulatory approval					

4.1. Is there a document relating to ethics committee approval and regulatory authority before the start of the study?					
4.2 4.2. Is there a system for reporting adverse events to the ethics committee and the regulatory authority? Reports of serious adverse events Responses from the ethics committee and the regulatory authority					
Highlights:					
Weak points					
Section 5: The trial site					
5.1 has it been approved or otherwise for conducting tests s? -Authorization of the extended authority; -The correct address of the authority; -The validity period of the authorization -The authenticity of					

the signatures and stamps of the authority; -Site registration.					
5.2 Does the site have the necessary equipment and structures for this? Presence of a space dedicated to physical examinations and equipped with the necessary and calibrated instruments; Controlled access at the clinic and pharmacy level; Presence of beds for hospitalization; Well-equipped emergency area with the necessary medications to administer in an emergency; Medicines to be administered in an emergency were stored according to their shelf life, and intervention equipment was appropriate.					
5.3. Does the site meet hygiene and biosecurity standards?					

Toilet and washing facilities were available; Presence of waste disposal system. Equipped kitchens and dining rooms.					
Section 6: Management of the pharmacy and the experimental product					
6.1. Are there procedures for managing titrants within the pharmacy? Records for entry and exit of inputs within the pharmacy. Standardized operating procedures* were detailed and described the different activities in the pharmacy. -Storage conditions were appropriate, as required, for the preservation of the products.					
6.2. Was the distribution and administration of experimental products carried out in accordance with a standardized operating procedure and randomization,					

without risk of confusion? The investigational medical products were correctly labeled: Randomization sheet and diagram					
Strong points					
Weak points:					
Section 7: Research staff					
7.1. Have all staff responsibilities and organizational chart been described? From the promoter; Investigators; Other staff; Organizational chart Evidence of delegation of tasks to staff based on their qualifications and experience was available					
7.2. Does the staff profile meet the requirements? Documents relating to qualifications including contractual staff; Appropriate experience and training;					

The number of employees was sufficient to carry out the test.					
7.3. Is there a monitoring-evaluation system?					
7.4. A monitoring manager with appropriate experience has been appointed to monitor the study					
Strong points:					
Weak points					
Section 8: Subject					
8.1. Is there a study information form? On the advantages of the test: -The experimental medical product; - Possible adverse events; - Insurance and other questions through the information form; - The contact details of the principal investigator or secretariat were communicated to the subjects					

8.2. Have all subjects participating in the study signed informed consent forms (ICF)? FCE contained all required information presented in a way that could be understood by the subjects. Translator; Good signed version;					
Strong points: Weak points:					
Section 9: Management of adverse events					
9.1 Is their consistency between the results and data recorded in the case report forms and those contained in the notification sheets? Presence of notification form; Presence of case report; Responsible for monitoring adverse effects; Therapeutic committee.					
9.2. How does your adverse event management system work?					

Samples, including blood and urine, were taken, chest x-rays or other tests have been carried out as required; Adverse events, concomitant medications, dosage, and sample collection were accurately recorded. Strong points: Weak points:					
Section 10: Analytical laboratory					
10.1. Were the laboratories adequately equipped to carry out the required tests? Automatons; The necessary equipment; Contracts in place for subcontracting; Compliance with sample storage and handling procedures; Equipment records were kept; The documents were appropriate for the qualification and calibration of laboratory equipment and instruments; Resources necessary to					

carry out the desired analyses; Qualification and calibration status was valid at the time of use for method validation and sample analysis.					
10.2. How are laboratory samples managed? Basic data was recorded accurately; The spaces intended for the reception and conservation (-20 or - 70° degrees Celsius until the time of analysis) of the samples, the preparation of the samples and the analysis were appropriate; The freezers used to store the samples were qualified; The files were traceable; The test methods were					

REPUBLIQUE DU CAMEROUN
Paix-Travail-Patrie

MINISTERE DE LA SANTE PUBLIQUE

REPUBLIC OF CAMEROON
Peace-Work-Fatherland

MINISTRY OF PUBLIC HEALTH

DECISION N° 4 9 8 / D/ MINSANTE/SG/DROS/DAJC DU 04 OCT 2023
rendant exécutoire les Lignes Directrices pour les Essais Cliniques au Cameroun.

LE MINISTRE DE LA SANTE PUBLIQUE,

Vu la Constitution ;
Vu la loi n°96/03 du 04 janvier 1996 portant loi cadre dans le domaine de la santé ;
Vu la loi n°2022/008 du 27 Avril 2022 relative à la recherche médicale impliquant la personne humaine au Cameroun ;
Vu le décret n°2011/408 du 9 décembre 2011 portant organisation du Gouvernement, modifié et complété par le décret n°2018/190 du 02 mars 2018 ;
Vu le décret n° 2013/093 du 03 avril 2013 portant organisation du Ministère de la Santé Publique ;
Vu le décret n° 2019/002 du 04 janvier 2019 portant réaménagement du Gouvernement ;
Vu l'arrêté N° 977/A/MINSANTE/SESP/SG/DROS du 18 avril 2012 portant création, organisation et fonctionnement des Comités d'Ethique de la Recherche pour la Santé Humaine au sein des structures relevant du ministère en charge de la santé publique ;
Vu l'arrêté N° 1737/ A/MINSANTE/SG/DROS du 28 novembre 2013 fixant les modalités des frais d'évaluation des protocoles de recherche soumis aux comités d'éthique de la recherche pour la santé humaine pour l'obtention d'une clairance éthique ;

Considérant les nécessités de service,

DECIDE:

ARTICLE 1.- La présente décision rend exécutoire les lignes directrices pour les essais cliniques au Cameroun.

ARTICLE 2.- Les lignes directrices susvisées sont jointes en annexe de la présente décision.

ARTICLE 3.- Les lignes directrices pour les essais cliniques au Cameroun feront l'objet d'une actualisation en tant que de besoin, à l'initiative du Ministre chargé de la Santé Publique.

ARTICLE 4.- La présente décision qui prend effet à compter de la date de signature, sera enregistrée puis communiquée partout où besoin sera, en français et en anglais.-/

Yaoundé, le _____

REPUBLIQUE DU CAMEROUN Republic of Cameroon PAIX-TRAVAIL-PATRIE Peace-Work-Fatherland LE MINISTRE The Minister Ministry of Public Health MINISTERE DE LA SANTE PUBLIQUE

LE MINISTRE DE LA SANTE PUBLIQUE,

Dr. MANAOUDA Malachie